

#1 · Management of Care

What are the five rights of delegation?

Answer

The five rights are the right task, right circumstance, right person, right direction or communication, and right supervision or evaluation. An LPN or RPN uses these rights to decide whether a task can be safely handed off to an unregulated care provider. All five must be met before delegation proceeds.

#2 · Management of Care

Can an LPN delegate a task to an unregulated care provider (UCP) and then walk away from the situation?

Answer

No, delegation always includes ongoing accountability. The LPN who delegates remains responsible for supervising the outcome and confirming the UCP has the knowledge and skill to complete the task safely. The LPN cannot transfer accountability for the nursing judgment involved, only the act itself.

#3 · Management of Care

When prioritizing four clients at the start of a shift, which client should the LPN assess first: one reporting new chest pain, one due for a scheduled dressing change, one asking for a snack, or one waiting for discharge teaching?

Answer

The client reporting new chest pain should be assessed first because it may signal a life-threatening cardiac event. Using an ABC and Maslow-based approach, physiological threats to airway, breathing, and circulation always outrank comfort requests, scheduled routine care, and teaching needs.

#4 · Management of Care

What is the difference between an LPN's scope of practice and their individual competence?

Answer

Scope of practice is the legal boundary set by provincial legislation and the regulatory college describing what LPNs as a group are authorized to do. Individual competence is whether a specific LPN has the education, skill, and current experience to perform a particular task safely. An LPN must stay within both the legal scope and their own personal competence before accepting an assignment.

#5 · Management of Care

A client tells the nurse a personal detail and asks that it not be shared with the family. What principle guides the nurse's response?

Answer

Confidentiality requires the nurse to protect client information and share it only with the interdisciplinary care team on a need to know basis. Information should not be disclosed to family members without the client's consent unless there is a legal duty to warn or a risk of serious harm. The nurse should document the client's wish and respect it within legal limits.

#6 · Management of Care

What are the three elements required for informed consent to be valid?

Answer

Valid consent requires that it be given voluntarily, by a capable person, and based on sufficient information about the treatment, its risks, benefits, and alternatives. The person must also have the capacity to understand and reason about that information. Consent obtained without any one of these elements is not legally valid.

#7 · Management of Care

Who is the appropriate substitute decision maker when a client loses capacity and has not named a representative in Ontario?

Answer

The Health Care Consent Act sets out a hierarchy that typically starts with a court-appointed guardian, then a person named in a power of attorney for personal care, then a spouse or partner, then a child or parent, followed by other relatives, and finally the Public Guardian and Trustee. The nurse works with the highest-ranking available person in this hierarchy to obtain consent.

#8 · Management of Care

What does client advocacy mean in daily LPN practice?

Answer

Advocacy means speaking up for the client's expressed wishes, rights, and safety, especially when the client cannot do so themselves. This can include questioning an order that seems unsafe, notifying the team of a change in condition, or supporting the client's right to refuse treatment. The nurse advocates based on the client's values, not the nurse's own opinion.

#9 · Management of Care

A competent adult client refuses a prescribed medication. What should the LPN do?

Answer

The LPN should respect the client's right to refuse treatment since a capable adult has the legal right to accept or decline care. The nurse documents the refusal, the information given to the client about the risks of refusing, and notifies the prescriber or charge nurse. The client should not be pressured or given the medication against their wishes.

#10 · Management of Care

What is the purpose of a case manager in continuity of care?

Answer

A case manager coordinates services across settings and providers so the client receives consistent, timely care as they move between hospital, home, and community. This includes arranging referrals, monitoring progress toward goals, and communicating updates among the care team. The goal is to avoid gaps or duplication in the client's care plan.

#11 · Management of Care

What information belongs in a proper discharge referral to a community agency?

Answer

A referral should include the client's diagnosis, current status, treatments in progress, medications, mobility and safety needs, and specific reason for the referral. It should also state what outcome or service is being requested from the receiving agency. Incomplete referrals can delay or interrupt the client's continuity of care.

#12 · Management of Care

What is the legal purpose of nursing documentation?

Answer

Documentation creates a permanent legal record of the care provided, the client's response, and the clinical reasoning behind decisions. It supports continuity of care between shifts and providers and can be used as evidence in a legal or regulatory review. The standard is that if care was not documented, it may be treated as if it was not done.

#13 · Management of Care

What should an LPN chart after a medication error occurs?

Answer

The nurse documents the objective facts: what medication was given, the dose, time, the client's assessment findings, and who was notified. An incident report is completed separately and is not referenced in the client's chart. The chart entry should never include blame, opinions, or the fact that an incident report was filed.

#14 · Management of Care

What is the difference between an ethical dilemma and a legal problem in nursing practice?

Answer

An ethical dilemma involves a conflict between two or more moral values where there is no single clearly right answer, such as respecting autonomy versus preventing harm. A legal problem involves a violation of law or regulation with a defined standard to measure against. A situation can be legal but still raise an ethical concern for the nurse.

#15 · Management of Care

What is the ethical principle of beneficence?

Answer

Beneficence is the duty to act in ways that benefit the client and promote their wellbeing. It requires the nurse to take positive steps to help, not just avoid harm. This principle guides decisions like advocating for pain relief or arranging needed support services.

#16 · Management of Care

What is the ethical principle of nonmaleficence?

Answer

Nonmaleficence is the duty to do no harm to the client. It requires the nurse to weigh the risks and benefits of an intervention before proceeding and to avoid actions that could injure the client physically or emotionally. This principle often works alongside beneficence when planning care.

#17 · Management of Care

During shift handover, which format is commonly used to organize the report between nurses?

Answer

SBAR, standing for situation, background, assessment, and recommendation, is a widely used structured format for handover communication. It helps the receiving nurse quickly understand the client's current status and what needs attention. Consistent use of a structured format reduces the risk of missed or miscommunicated information.

#18 · Management of Care

What is a nurse's legal duty when they witness unsafe or unethical practice by a colleague?

Answer

The nurse has a professional obligation to report the concern through the appropriate channel, such as a supervisor, manager, or the regulatory college if the situation is serious. Staying silent can make the observing nurse professionally accountable as well. The duty exists to protect client safety, not to punish the colleague.

#19 · Management of Care

When assigning care using the ABC framework, which client problem takes priority: a blocked airway or low blood pressure?

Answer

A blocked airway takes priority because airway problems threaten life within minutes and must be corrected before breathing or circulation issues are addressed. The ABC framework orders priorities as airway first, then breathing, then circulation. Low blood pressure is addressed immediately after the airway is secured.

#20 · Management of Care

What does 'continuity of care' mean for a client being transferred from hospital to a long term care facility?

Answer

Continuity of care means the client's treatment plan, medications, and goals remain consistent and are clearly communicated to the receiving facility. This prevents gaps such as missed medications or repeated assessments. A complete transfer summary and verbal handover both support this continuity.

#21 · Management of Care

Can an LPN in Ontario perform controlled acts such as administering medication by injection?

Answer

Yes, LPNs can perform certain controlled acts, including giving medications by injection, when authorized by the Regulated Health Professions Act and their employer's policies, and when the LPN has the required education and demonstrated competence. This authority applies within the LPN's defined scope, not without limits. Some higher-risk controlled acts still require an RN or other authorized provider.

#22 · Management of Care

What should an LPN do if asked to perform a task outside their personal competence, even if it falls within the legal scope of practice?

Answer

The LPN should decline the task and communicate the competence gap to the supervisor or charge nurse rather than attempting it. Accepting an assignment beyond one's competence puts the client at risk and can be considered professional misconduct. The nurse can request supervision, education, or reassignment of the task instead.

#23 · Management of Care

What is the purpose of an interdisciplinary care conference?

Answer

An interdisciplinary care conference brings together nursing, medicine, social work, rehabilitation, and other relevant providers to review the client's progress and update the care plan together. It allows the client's goals to be addressed from multiple professional perspectives at once. This collaboration supports coordinated, client-centred continuity of care.

#24 · Management of Care

What is the difference between assault and battery in the context of client care?

Answer

Assault is the threat or attempt to touch a client without consent that causes the client to fear imminent harm, while battery is the actual unwanted physical contact. Both can occur if a nurse performs a procedure after a client has clearly refused. Obtaining consent before any procedure prevents both types of claims.

#25 · Management of Care

Using Maslow's hierarchy, which need is addressed before a client's need for social interaction or teaching?

Answer

Physiological needs such as oxygen, nutrition, elimination, and pain control are addressed first, followed by safety and security needs. Social and love and belonging needs, including interaction and emotional support, come after basic physiological and safety needs are met. Teaching is generally lower priority than an unmet physical or safety need.

#26 · Management of Care

What should an LPN do when a client's family member insists on receiving detailed medical information but the client has not consented to share it with them?

Answer

The LPN should explain that health information is confidential and cannot be shared without the client's consent, per privacy legislation such as PHIPA in Ontario. The nurse can encourage the family member to discuss the matter directly with the client. General reassurance can be offered without disclosing specific clinical details.

#27 · Management of Care

Under the Five Rights of Delegation, what does the right of "supervision" actually require the delegating nurse to do?

Answer

The delegating nurse must provide appropriate direction, monitor the task as it is being performed, and evaluate the outcome once it is complete. Supervision is not a one time check at the start; it continues until the task is finished and the result is confirmed safe. The delegating nurse remains accountable for the decision to delegate even though the person completing the task owns the act itself.

#28 · Management of Care

An unregulated care provider (UCP) reports a client's oxygen saturation as 88 percent. Whose responsibility is it to interpret that finding and decide on action?

Answer

The regulated nurse who delegated the vital sign measurement is responsible for interpreting the value and deciding what to do next. UCPs can collect data but cannot analyze or act on abnormal findings independently. The nurse must assess the client directly before any change in care is made.

#29 · Management of Care

What is the main difference between accountability and responsibility in nursing delegation?

Answer

Responsibility can be shared or transferred to the person performing a delegated task, but accountability for the overall outcome always stays with the regulated nurse who made the delegation decision. A nurse can hand off the doing of a task, but never the obligation to answer for whether delegating it was appropriate. This distinction is tested heavily on assignment and delegation questions.

#30 · Management of Care

When prioritizing four clients at the start of a shift, which framework should be applied first: Maslow's hierarchy or the ABC (airway, breathing, circulation) approach?

Answer

Airway, breathing, and circulation problems always take priority over Maslow's hierarchy because they represent immediate threats to life. Once ABC issues are ruled out or stabilized, Maslow's hierarchy can guide prioritization among remaining physiologic, safety, and psychosocial needs. A client with a partially obstructed airway outranks a client expressing anxiety even though anxiety sits on Maslow's pyramid.

#31 · Management of Care

What distinguishes a sentinel event from a near miss in client safety reporting?

Answer

A sentinel event is an unexpected occurrence that results in death or serious physical or psychological injury to the client. A near miss is an error that was caught before it reached the client and caused no harm. Both should be reported through the facility's incident reporting system, but a sentinel event typically triggers a root cause analysis.

#32 · Management of Care

A client tells the LPN/RPN a personal secret and asks that it never be shared with the rest of the care team. How should the nurse respond?

Answer

The nurse should explain that information relevant to the client's care must be shared with the interprofessional team to provide safe treatment, so complete confidentiality from the team cannot be promised. Confidentiality applies to protecting information from people outside the circle of care, not from other providers who need it to deliver care. The nurse should still limit what is shared to what is clinically necessary.

#33 · Management of Care

What is the legal difference between implied consent and informed consent?

Answer

Implied consent is inferred from a client's actions, such as holding out an arm for a blood pressure cuff, and applies mainly to routine, low risk care. Informed consent requires that the client receive an explanation of the procedure, its risks, benefits, and alternatives before agreeing, and is required for treatments carrying meaningful risk. A signature alone does not make consent informed if the explanation never happened.

#34 · Management of Care

Who is legally responsible for obtaining informed consent before a surgical procedure?

Answer

The physician or other provider who will perform the procedure is responsible for obtaining informed consent, because only they can fully explain the risks, benefits, and alternatives. The nurse's role is to witness the client's signature, confirm the client appears to understand, and notify the provider if the client expresses doubt or confusion. A nurse should never obtain informed consent on the provider's behalf for a procedure the nurse is not performing.

#35 · Management of Care

In the context of continuity of care, what is the purpose of a structured handoff tool such as SBAR?

Answer

SBAR (situation, background, assessment, recommendation) organizes the information a nurse gives during a handoff so nothing critical is left out during a transfer of care. It reduces the risk of communication errors, which are a leading cause of preventable harm during shift change or transfer between units. Using a consistent structure also lets the receiving nurse anticipate what information comes next.

#36 · Management of Care

What is the LPN/RPN's role when a client wants to refuse a treatment that the LPN/RPN believes is medically necessary?

Answer

The nurse must respect the competent client's right to refuse treatment even if the nurse disagrees with the decision, since capable adults have the right to autonomy over their own bodies. The nurse should document the refusal, the education given about the risks of refusing, and notify the physician or supervising provider. The nurse cannot force treatment on a capable client who has refused it.

#37 · Management of Care

What must be true for a client's refusal of treatment to be considered valid?

Answer

The client must have the mental capacity to understand the information given, appreciate the consequences of refusing, and communicate a clear decision voluntarily and without coercion. Capacity is assessed for each specific decision, not as an all or nothing global status. A client can lack capacity for one complex decision while retaining capacity for simpler ones.

#38 · Management of Care

What is the difference between an incident report and the client's health record when documenting a medication error?

Answer

The health record should contain an objective, factual description of what happened, what assessment was done, and what interventions followed the error, without referencing that an incident report was filed. The incident report is an internal quality improvement document used for facility review and is not part of the client's permanent chart. Mentioning the incident report inside the chart can create legal complications if the record is later reviewed.

#39 · Management of Care

When should late entry documentation be used, and how should it be labeled?

Answer

A late entry is used when the nurse forgot to document care at the time it occurred and needs to add it after the fact. The entry should be labeled clearly as "late entry," dated and timed with the current date and time, and should state the actual date and time the care was originally given. The original omission should never be backdated to look like it was written at the time of care.

#40 · Management of Care

What is the correct way to correct a documentation error made in a paper chart?

Answer

The nurse should draw a single line through the incorrect entry so the original text is still readable, write the correction beside or above it, then add the date, time, and initials. Erasing, using correction fluid, or blacking out the error is not acceptable because the chart is a legal document that must show its full history. The goal is transparency, not concealment of the mistake.

#41 · Management of Care

A client asks the LPN/RPN to be their advocate in a disagreement with family about discharge plans. What does client advocacy require the nurse to do in this situation?

Answer

Advocacy means representing the client's own expressed wishes and rights to the rest of the team, not substituting the nurse's personal opinion about the best plan. The nurse should make sure the client's voice is heard in team discussions and that the client has the information needed to make their own choice. Advocacy does not mean overriding the family or making the decision for the client.

#42 · Management of Care

In Ontario and British Columbia, can an LPN/RPN independently develop and revise a client's overall plan of care without any input from an RN?

Answer

LPNs/RPNs can contribute to and update parts of a care plan within their scope, particularly for clients with predictable, stable conditions. However, for complex or unstable clients, collaboration with an RN or the broader team is expected, since scope of practice ties independent care planning to the predictability of the client's condition. Facility policy also plays a role in defining exactly how much independent planning an LPN/RPN can do.

#43 · Management of Care

What is the key factor that determines whether a task can safely be assigned to an LPN/RPN rather than requiring an RN?

Answer

The key factor is the stability and predictability of the client's condition, not simply the type of task itself. A stable client needing a task that is technically complex may still be appropriate for an LPN/RPN, while an unstable client needing a simple task may require RN level judgment. Assignment decisions are based on matching the caregiver's competency to the client's level of complexity and risk.

#44 · Management of Care

What should an LPN/RPN do if asked to perform a procedure that is outside their individual competency, even though it is technically within the LPN/RPN scope of practice in general?

Answer

The nurse must decline to perform the procedure and inform the supervisor, because scope of practice sets the outer legal boundary but personal competency determines what an individual nurse can safely do. Performing a task without the skill or recent experience to do it safely puts the client at risk and the nurse at professional liability. The nurse should request supervision, additional training, or reassignment of the task.

#45 · Management of Care

When making a referral to another service, such as social work or physiotherapy, what information must the referring nurse include to make the referral appropriate?

Answer

The referral should include the specific reason for the referral, relevant assessment findings that support the need, and any urgency or time sensitivity involved. Vague referrals without supporting data can delay care because the receiving service cannot prioritize appropriately. The nurse should also confirm the client has agreed to the referral where consent is required.

#46 · Management of Care

What is the difference between case management and care coordination as used in Canadian practice settings?

Answer

Case management typically refers to a structured, often longer term process of assessing, planning, and monitoring a client's overall needs across services, frequently associated with a designated case manager role. Care coordination is the broader, ongoing act of organizing communication and services among providers so care is not fragmented, and it happens as part of everyday nursing practice. Every nurse participates in care coordination, but not every nurse holds a formal case management role.

#47 · Management of Care

A client with dementia who lacks capacity for a specific decision needs a treatment consent. Who provides consent in this situation under Canadian health law principles?

Answer

Consent must be obtained from the client's substitute decision maker, following the hierarchy set out in provincial legislation, which typically starts with a court appointed guardian, then a named power of attorney for personal care, then family members in a specified order. The substitute decision maker must make the decision based on the client's previously expressed wishes if known, or the client's best interests if wishes are unknown. The nurse should confirm who the legally recognized substitute decision maker is before treatment proceeds.

#48 · Management of Care

What ethical principle is primarily being applied when a nurse allocates limited resources, such as staff time during short staffing, based on which clients have the most urgent needs?

Answer

This reflects the principle of justice, which involves the fair distribution of limited resources according to need. Justice does not always mean equal distribution; it means allocating resources in a way that is fair given differing levels of need and risk. This principle is commonly tested in disaster triage and short staffing scenario questions.

#49 · Management of Care

What is the difference between beneficence and nonmaleficence as ethical principles in nursing practice?

Answer

Beneficence is the obligation to actively do good and act in the client's best interest, while nonmaleficence is the obligation to avoid causing harm. A nurse practicing beneficence might advocate for a treatment that will help a client, while nonmaleficence would stop the nurse from performing an action likely to cause more harm than benefit. The two principles can come into tension, such as when a beneficial treatment carries a real risk of harm.

#50 · Management of Care

A colleague asks the LPN/RPN to falsify a witness signature on a consent form because the original witness already left the unit. What should the nurse do?

Answer

The nurse must refuse, since signing as a witness to something not personally observed is falsification of a legal document and a serious professional conduct violation. The correct action is to have the actual witness sign later if possible, or to have a person who is currently present witness the signature honestly. The nurse should also be aware this kind of request may need to be reported depending on facility policy.

#51 · Management of Care

What is the purpose of a critical pathway or clinical pathway document in coordinating a client's care?

Answer

A critical pathway is a standardized, evidence based outline of the expected sequence and timing of interventions for a specific condition or procedure, used to keep the care team aligned on goals and timelines. It helps identify variances early when a client's progress does not match the expected trajectory, prompting the team to investigate why. It supports continuity of care by giving every provider a shared reference point for what should happen next.

#52 · Management of Care

When a client is being transferred from one facility to another, what is the LPN/RPN's minimum documentation responsibility before the transfer occurs?

Answer

The nurse must document the client's current status, the reason for transfer, the mode of transport, and confirmation that the receiving facility or unit was given a report before the client left. This ensures a clear legal record that continuity of care was maintained during the transition. Missing this documentation can create liability if a gap in care is later identified.

#53 · Management of Care

What are the five rights of delegation that an LPN or RPN must confirm before accepting a delegated task?

Answer

The five rights are right task, right circumstance, right person, right direction and communication, and right supervision and evaluation. The delegating nurse checks each right before assigning a task to an unregulated care provider. If any right is missing, the task should not be delegated at that time.

#54 · Management of Care

Can an LPN or RPN delegate a controlled act to an unregulated care provider?

Answer

Yes, in most Canadian provinces a regulated nurse can delegate certain controlled acts, such as administering a medication by a specific route, if the provincial regulator and employer policy allow it. The nurse must assess the client's stability, the care provider's competency, and provide clear direction. The delegating nurse remains accountable for the decision to delegate and for ongoing supervision.

#55 · Management of Care

Using SBAR, what does the S stand for and what should it include?

Answer

S stands for Situation, which is a brief statement of the client's current problem or the reason for the call. It should include the client's name, location, and a concise description of what is happening right now. This orients the listener immediately to why the communication is urgent or important.

#56 · Management of Care

During an SBAR handoff, what information belongs in the Background section?

Answer

Background includes relevant history such as admitting diagnosis, date of admission, current medications, allergies, and recent relevant test results. It gives the listener context for understanding why the situation is happening. It should stay focused on facts that directly relate to the current concern.

#57 · Management of Care

A client tells the LPN a secret about a family conflict and asks that it not be shared with anyone, including the healthcare team. What should the nurse do?

Answer

The nurse should explain that information relevant to the client's care cannot be kept from the healthcare team because safe care depends on shared documentation and communication. The nurse can reassure the client that personal information is only shared on a need to know basis for care purposes. Confidentiality has limits when disclosure is needed to protect the client's safety or the safety of others.

#58 · Management of Care

What is the correct order of prioritization when a nurse has four clients needing attention at the start of a shift?

Answer

The nurse should use frameworks such as ABC (airway, breathing, circulation) and Maslow's hierarchy of needs, prioritizing the client with the most immediate threat to life first. A client with an airway or breathing problem takes priority over a client with a stable chronic condition. Unstable clients or those with sudden changes in condition are always assessed before routine or stable tasks.

#59 · Management of Care

What is informed consent and what three elements must be present for it to be valid?

Answer

Informed consent is a client's voluntary agreement to a treatment or procedure after understanding its nature, risks, benefits, and alternatives. The three required elements are disclosure of relevant information, the client's capacity to understand and decide, and voluntariness free from coercion. Consent obtained without all three elements is not legally valid.

#60 · Management of Care

Who is legally responsible for obtaining informed consent for a surgical procedure?

Answer

The healthcare provider who will perform the procedure, such as the surgeon, is responsible for obtaining informed consent. The LPN or RPN can witness the client's signature and confirm the client's understanding, but cannot legally provide the detailed procedural explanation or obtain consent on the provider's behalf. If the nurse discovers the client does not understand the procedure, the nurse must notify the provider before the procedure proceeds.

#61 · Management of Care

What is the nurse's role as a client advocate when a client refuses a recommended treatment?

Answer

The nurse respects the client's autonomous right to refuse treatment as long as the client has decision-making capacity and understands the consequences of refusal. The nurse documents the refusal, notifies the healthcare provider, and continues to offer support and education without pressuring the client. Advocacy means protecting the client's right to choose, not overriding it with the nurse's own opinion.

#62 · Management of Care

What does continuity of care mean in the context of client transfers between units or facilities?

Answer

Continuity of care means the client receives consistent, coordinated care as they move between caregivers, units, or healthcare settings, without gaps or duplication in treatment. It relies on accurate handoff communication, complete documentation, and a shared care plan. Poor continuity increases the risk of medication errors, missed treatments, and client harm.

#63 · Management of Care

An LPN receives a verbal order from a physician over the phone. What must happen before the LPN carries out the order?

Answer

The LPN must read the order back to the physician to confirm accuracy, a practice known as read-back verification. The order should be documented as a verbal order with the date, time, and prescriber's name, and cosigned by the prescriber according to facility policy within the required timeframe. Verbal orders should only be accepted in urgent situations where a written order is not immediately possible.

#64 · Management of Care

What is the purpose of an electronic health record's audit trail feature?

Answer

An audit trail automatically logs who accessed a client's record, when, and what changes were made, creating accountability for documentation practices. It helps detect unauthorized access to confidential health information. Nurses should only access records for clients they are directly caring for, since inappropriate access can be tracked and disciplined.

#65 · Management of Care

What is the difference between assignment and delegation in nursing practice?

Answer

Assignment is giving a task to someone who already has the authority and scope of practice to perform it, such as one regulated nurse assigning a task to another regulated nurse. Delegation is transferring authority to perform a task to someone who would not otherwise have that authority, such as a regulated nurse delegating a task to an unregulated care provider. The accountability structure differs because delegation always requires ongoing supervision by the delegating nurse.

#66 · Management of Care

Within the LPN or RPN scope of practice, what type of client is generally most appropriate to assign to an LPN in an acute care setting?

Answer

LPNs and RPNs are generally assigned clients whose conditions are stable and predictable, where outcomes can be reasonably anticipated based on the client's history and current status. Clients who are unstable, complex, or rapidly changing are typically assigned to a registered nurse with broader scope and additional assessment skills. Provincial regulators and employer policy determine the exact boundaries, and the nurse must self-assess competency for each assignment.

#67 · Management of Care

What should an LPN do if asked to perform a task that is outside their individual competence, even though it is within the legal scope of practice for LPNs generally?

Answer

The nurse must decline the task and inform the supervisor, because legal scope of practice sets the outer boundary but individual competence determines what a specific nurse can safely perform. Practising beyond individual competence puts the client at risk and can be considered professional misconduct. The nurse should request additional training or supervision before attempting the task again.

#68 · Management of Care

What is the nurse's responsibility when making a referral to another healthcare professional, such as a dietitian or social worker?

Answer

The nurse identifies the client's need, communicates relevant clinical information to the receiving professional, and documents the referral in the client's record. The nurse should confirm the client agrees to the referral when appropriate and follow up to ensure the referral was completed. Referrals support interprofessional collaboration and help address needs outside the nurse's own scope of practice.

#69 · Management of Care

What is the correct action if a nurse notices an error made by another healthcare team member on a client's chart?

Answer

The nurse should not delete or erase the entry, and instead follow facility policy for correcting documentation errors, which usually involves the original author drawing a single line through the error, initialing it, and writing the correct information. If the error affects client safety, the nurse should also notify the appropriate person immediately. Falsifying or altering another provider's documentation is not acceptable practice.

#70 · Management of Care

What ethical principle is being applied when a nurse divides limited staffing resources fairly among all clients on a unit?

Answer

This reflects the principle of justice, which means treating people fairly and distributing resources, care, and attention equitably based on need. Justice does not always mean equal distribution, since clients with greater needs may require more resources. It guides decisions such as staffing allocation and priority setting during resource shortages.

#71 · Management of Care

What is the difference between the ethical principles of beneficence and nonmaleficence?

Answer

Beneficence means actively doing good and acting in the client's best interest, such as providing comfort measures or advocating for needed treatment. Nonmaleficence means avoiding harm to the client, such as double checking a medication dose before administration. Both principles guide nursing decisions, but nonmaleficence sets a baseline of avoiding harm while beneficence goes further to actively promote wellbeing.

#72 · Management of Care

A client asks the LPN to explain their diagnostic test results in detail. What is the appropriate response?

Answer

The LPN can review results that are already documented and explained by the provider, and can reinforce teaching within their scope, but should not interpret new or complex results beyond their competence or the provider's explanation. The nurse should notify the provider if the client has questions requiring clinical interpretation. This protects the client from receiving inaccurate or premature information.

#73 · Management of Care

What is the purpose of a standardized handoff tool such as SBAR during a change of shift report?

Answer

A standardized handoff tool ensures critical client information is communicated consistently and completely between caregivers, reducing the risk of missed or miscommunicated information. It structures the report into predictable sections so the receiving nurse knows what to expect and can ask focused questions. Standardized handoffs are linked to fewer adverse events and improved continuity of care.

#74 · Management of Care

When is it appropriate for an LPN or RPN to accept a client assignment through team nursing versus total client care?

Answer

In team nursing, an LPN works under the direction of a registered nurse team leader who assigns tasks based on each team member's scope and competency, which is appropriate for clients with complex or changing needs. In total client care, the LPN independently manages all aspects of care for stable clients within their full scope of practice. The choice of care delivery model depends on client acuity, staffing, and facility policy.

#75 · Management of Care

What should a nurse do if they disagree with a physician's order because they believe it could harm the client?

Answer

The nurse should not carry out the order and must first clarify the order directly with the prescriber, explaining the specific concern. If the concern is not resolved, the nurse follows the chain of command, notifying a supervisor or manager, and documents the concern and actions taken. The nurse has a professional and legal obligation to advocate for client safety even when it means questioning a prescriber's order.

#76 · Management of Care

What information is essential to include when documenting a client incident, such as a fall?

Answer

Documentation should include the objective facts of what was observed, the client's condition before and after the event, actions taken, and who was notified, such as the physician or family. The nurse should avoid including blame, opinions, or assumptions about cause. A separate incident report is usually completed according to facility policy, but it is not referenced in the client's chart.

#77 · Management of Care

Why is it important for an LPN or RPN to use only approved abbreviations when documenting client care?

Answer

Approved abbreviation lists reduce the risk of misinterpretation that could lead to medication errors or unsafe care, since some abbreviations look similar or have multiple meanings. Many regulators and accreditation bodies maintain a 'do not use' list of dangerous abbreviations. Consistent, approved documentation supports clear communication across the entire healthcare team.

#78 · Management of Care

What is the LPN or RPN's obligation regarding client confidentiality when using electronic devices such as personal phones on a unit?

Answer

Nurses must not photograph, record, or share any client information or images using personal devices, since this violates privacy legislation and professional standards of confidentiality. Client information should only be accessed and documented through approved, secure facility systems. Breaching confidentiality through personal devices can result in legal consequences and disciplinary action by the regulatory body.

#79 · Management of Care

When using the ABC framework to prioritize four clients at once, which client does the LPN assess first?

Answer

The client with an airway problem is assessed first, since airway takes priority over breathing, circulation, and disability. A client who is choking or has stridor outranks one who is simply short of breath. This ordering exists because airway loss causes death fastest of the four categories.

#80 · Management of Care

A postoperative client reports new chest tightness while another client's dressing has light drainage. Who does the LPN see first?

Answer

The client with new chest tightness is seen first because it may signal a cardiac or respiratory emergency, which falls under circulation and breathing on the ABC scale. A dressing with light drainage is a lower-acuity finding that can wait a few minutes. Prioritization always favors the client whose condition could deteriorate fastest.

#81 · Management of Care

How does Maslow's hierarchy of needs guide an LPN when several clients request help at the same time?

Answer

Physiological needs such as oxygen, hydration, and pain relief are met before safety needs, and safety needs are met before love, esteem, or self-actualization needs. For example, a client needing suctioning is prioritized over a client asking for information about a support group. The lower, more basic need on the pyramid is addressed first.

#82 · Management of Care

A client refuses a scheduled medication. What must the LPN do before documenting the refusal?

Answer

The LPN must explain the purpose and risks of refusing the medication so the client can make an informed decision, then respect the client's right to refuse. The LPN documents the education given, the client's stated reason if offered, and notifies the prescriber or supervising nurse as needed. Forcing the medication would violate the client's autonomy.

#83 · Management of Care

What four elements must be present for a client's consent to a treatment to be legally valid?

Answer

Consent must be voluntary, given by a client who has the capacity to understand the decision, specific to the treatment being proposed, and informed with enough information about risks, benefits, and alternatives. Missing any one of these elements makes the consent invalid. Capacity is decision-specific, so a client can consent to one treatment but lack capacity for a more complex one.

#84 · Management of Care

A 15-year-old client wants treatment for a sexually transmitted infection without a parent knowing. Can the LPN proceed?

Answer

In most Canadian provinces, a minor who demonstrates capacity to understand the nature and consequences of the treatment can consent under the mature minor doctrine, and there is no fixed legal age tied to capacity. The LPN assesses the minor's understanding rather than assuming incapacity based on age alone. Confidentiality is maintained unless there is a safety risk that requires disclosure.

#85 · Management of Care

An unconscious client arrives in acute distress with no substitute decision maker present. What consent rule applies?

Answer

Emergency implied consent allows the health care team to provide treatment necessary to preserve life or prevent serious harm without written consent. This applies only when the client cannot communicate and no immediate substitute decision maker is available. Once the emergency passes or a decision maker is reached, standard consent rules resume.

#86 · Management of Care

Who becomes the substitute decision maker for a client who loses capacity and has no advance directive naming one?

Answer

Provincial legislation sets a hierarchy that usually starts with a court-appointed guardian, then a spouse or partner, then children, parents, siblings, and other relatives, depending on the province. The LPN should know the local hierarchy rather than assume the closest family member automatically has authority. The substitute decision maker must make choices based on the client's known wishes or best interests.

#87 · Management of Care

What is the correct way for an LPN to delegate a task to an unregulated care aide?

Answer

The LPN must confirm the task is within the aide's training, assign it clearly, provide any necessary instructions, and supervise the outcome while remaining accountable for the decision to delegate. Tasks requiring nursing judgment, such as assessment or medication administration, cannot be delegated. The LPN who delegates a task remains professionally responsible for that decision.

#88 · Management of Care

Under LPN/RPN scope of practice, which client is the most appropriate assignment for an LPN working with a registered nurse team?

Answer

A stable client with a predictable, well-established plan of care is the most appropriate assignment for an LPN. Unstable clients, clients requiring complex assessment or initial care planning, and clients with unpredictable outcomes are better matched to a registered nurse. Assignment decisions should match client acuity and predictability to the nurse's scope and competence.

#89 · Management of Care

A family member calls the unit asking for details about a client's diagnosis. What should the LPN do first?

Answer

The LPN must confirm the client has consented to share health information with that specific person before disclosing anything. Confidentiality applies even to close family members unless the client has authorized disclosure or a substitute decision maker is legally acting on the client's behalf. If consent has not been confirmed, the LPN should decline to share details and offer to have the client call back.

#90 · Management of Care

What is the difference between confidentiality and privacy in a client care context?

Answer

Privacy refers to a client's right to control access to their own body, personal space, and presence during care activities. Confidentiality refers to the protection of health information that has already been collected. A curtain drawn during a physical exam protects privacy, while locking a chart protects confidentiality.

#91 · Management of Care

How should an LPN document a medication error that has already reached the client?

Answer

The LPN documents the facts objectively, including what was given, the time, the client's response, and who was notified, without writing opinions or blame. The prescriber and supervising nurse are notified immediately so the client can be assessed and monitored. A separate incident report is completed according to facility policy, but it is not referenced in the client's chart.

#92 · Management of Care

What is the purpose of a case manager in continuity of care?

Answer

A case manager coordinates services across providers and settings so that a client's care plan stays consistent as the client moves between hospital, home, and community resources. This includes arranging referrals, monitoring progress toward goals, and preventing duplicated or conflicting treatments. Good case management reduces gaps that lead to readmission or missed follow-up.

#93 · Management of Care

A client being discharged home after a stroke has new mobility limits. What should the LPN arrange before discharge?

Answer

The LPN should ensure referrals to physiotherapy, occupational therapy, and home care services are in place, and that the client and family understand follow-up appointments and warning signs to report. A discharge summary should be communicated to the receiving care provider to maintain continuity. Without these referrals, the client is at higher risk of falls or unmet care needs at home.

#94 · Management of Care

What ethical principle is being applied when an LPN advocates for a client's wish to stop aggressive treatment?

Answer

This reflects the principle of autonomy, which respects a capable client's right to make decisions about their own care, including refusing treatment. The LPN's role as advocate is to make sure the client's voice is heard by the care team, not to override it. Autonomy applies as long as the client has decision-making capacity.

#95 · Management of Care

A client with capacity asks the LPN to keep a new diagnosis secret from their spouse. What should the LPN do?

Answer

The LPN must respect the client's autonomy and confidentiality request, since the client has the right to decide who receives their health information. The LPN should not disclose the diagnosis to the spouse without the client's permission. The LPN can explore the client's reasons and offer support, but the final decision belongs to the client.

#96 · Management of Care

What is the ethical difference between beneficence and nonmaleficence in nursing practice?

Answer

Beneficence means actively acting to benefit the client, such as providing comfort measures or advocating for pain relief. Nonmaleficence means avoiding actions that could cause harm, such as double-checking a medication dose before administration. Both principles guide care, but nonmaleficence is often described as the duty to do no harm first.

#97 · Management of Care

An LPN disagrees with a care decision but the client, who has capacity, insists on it. What is the correct action?

Answer

The LPN respects the client's autonomous decision after ensuring the client is fully informed of the risks and benefits involved. The LPN can document their professional concern and continue to provide safe, nonjudgmental care. Overriding a capable client's wishes would violate their right to self-determination.

#98 · Management of Care

During a mass casualty triage, which category does a client with an open airway, respirations under 30, and no palpable radial pulse receive?

Answer

This client is triaged as immediate or red, because absent or delayed capillary refill and weak or absent pulses indicate impending shock even though the airway and breathing are currently adequate. Triage systems such as START prioritize circulation status alongside airway and breathing. Immediate category clients need life-saving intervention right away.

#99 · Management of Care

How should an LPN prioritize care when one client has a new fever of 38.9 and another has unrelieved pain rated 8 out of 10?

Answer

The new fever should generally be assessed first if it could indicate an emerging infection or sepsis, since an unexpected change in vital signs signals a potential decline. However, severe unrelieved pain also needs prompt attention and cannot be delayed indefinitely. The LPN uses clinical judgment to determine which finding represents a greater risk of rapid deterioration in that specific situation.

#100 · Management of Care

What should an LPN do if a physician's order seems unclear or potentially unsafe for a client?

Answer

The LPN must clarify the order directly with the prescriber before carrying it out, rather than guessing at the intent or skipping the task. If the order remains unsafe after clarification, the LPN follows the facility's chain of command to raise the concern further. Administering an unclear or unsafe order without question would place the client at risk and the LPN at professional liability.

#101 · Management of Care

What does the principle of justice mean in the context of allocating limited nursing time among assigned clients?

Answer

Justice means distributing care fairly based on each client's actual needs rather than favoring clients who are easier to work with or more vocal. An LPN practicing justice gives adequate attention to a quiet, confused client just as much as to one who frequently uses the call bell. Fair allocation of time and resources is part of ethical nursing practice.

#102 · Management of Care

A client with dementia and no documented advance directive needs a decision made about a feeding tube. Who decides?

Answer

Since the client lacks capacity for this decision, the legally recognized substitute decision maker is asked to decide based on the client's previously expressed wishes if known, or the client's best interests if wishes are unknown. The health care team provides the substitute decision maker with full information about risks and benefits. The decision must reflect what the client would have wanted, not simply what is easiest for the family.

#103 · Management of Care

Why is accurate and timely documentation considered part of client safety rather than just a legal formality?

Answer

Documentation communicates the client's current status and care given to every other provider who reads the chart, so gaps or errors can lead to repeated assessments, missed changes in condition, or medication mistakes. Timely documentation also creates a reliable record if a client's condition changes suddenly. Legal protection is a secondary benefit of documentation that is primarily done to support safe, continuous care.

#104 · Management of Care

An LPN notices a colleague has been charting medications before actually giving them. What is the appropriate response?

Answer

The LPN should address the concern directly with the colleague first if safe to do so, and report the practice to a supervisor since charting before administration falsifies the record and endangers client safety. This behavior can lead to a client receiving a missed dose or a duplicate dose if the medication is never actually given. Professional obligation requires reporting unsafe practice through proper channels rather than ignoring it.

#105 · Management of Care

What is the LPN's responsibility when a client's condition falls outside their current scope of practice or competence?

Answer

The LPN must recognize the limits of their own competence and promptly notify a registered nurse or supervisor rather than attempting care beyond their training. Continuing to provide care outside one's scope can result in client harm and professional liability. Reporting early allows a more qualified provider to reassess and adjust the care plan before the client's condition worsens.

#106 · Safety and Infection Control

What is the correct order for putting on personal protective equipment (PPE)?

Answer

The correct order is hand hygiene, gown, mask or respirator, goggles or face shield, then gloves. Gloves go on last so they cover the cuffs of the gown, and this sequence keeps the nurse hands clean the longest before contact with the patient.

#107 · Safety and Infection Control

What is the correct order for removing personal protective equipment (PPE)?

Answer

The correct order is gloves, goggles or face shield, gown, then mask or respirator. Gloves are removed first because they are the most contaminated item, and the mask is removed last since it protects the airway until the nurse is away from the patient.

#108 · Safety and Infection Control

Which type of precautions is used for a client with active pulmonary tuberculosis?

Answer

Airborne precautions are used, which require a negative pressure room and an N95 or higher respirator for anyone entering. Tuberculosis spreads through tiny droplet nuclei that stay suspended in the air, so a standard surgical mask does not provide enough protection.

#109 · Safety and Infection Control

Which type of precautions is required for a client with influenza?

Answer

Droplet precautions are required, which include a surgical mask, a private room or spacing of at least 2 metres, and standard precautions. Influenza spreads through large respiratory droplets that travel only a short distance before settling, unlike airborne pathogens.

#110 · Safety and Infection Control

Which type of precautions is used for a client infected with *Clostridioides difficile* (C. diff)?

Answer

Contact precautions are used, with a gown and gloves for anyone entering the room. Soap and water hand hygiene is required instead of alcohol-based hand rub because *C. diff* forms spores that alcohol does not reliably kill.

#111 · Safety and Infection Control

What are the three main types of transmission-based precautions?

Answer

The three types are contact, droplet, and airborne precautions. Each one is layered on top of standard precautions and is chosen based on how the specific organism spreads between people.

#112 · Safety and Infection Control

What is included in standard precautions?

Answer

Standard precautions include hand hygiene, use of PPE when contact with body fluids is expected, safe injection practices, respiratory hygiene and cough etiquette, and safe handling of contaminated equipment and linen. They are applied to every client, every time, regardless of known infection status.

#113 · Safety and Infection Control

When is alcohol-based hand rub not an appropriate choice for hand hygiene?

Answer

Alcohol-based hand rub should not be used when hands are visibly soiled or when caring for a client with *C. diff* or norovirus. In these situations, soap and water is required because alcohol does not remove spores or physical debris effectively.

#114 · Safety and Infection Control

What is the minimum recommended duration for washing hands with soap and water?

Answer

Hands should be washed with soap and water for at least 15 to 20 seconds, covering all surfaces including between the fingers, thumbs, and under the nails. This duration is long enough to mechanically remove most transient microorganisms.

#115 · Safety and Infection Control

What is the correct technique for using a mechanical lift to transfer a client with limited mobility?

Answer

The nurse should check the sling and lift for damage, position the sling correctly under the client, ensure the client's arms stay inside the sling straps, and use a minimum of two staff members during the transfer. This reduces the risk of falls for the client and musculoskeletal injury for the caregiver.

#116 · Safety and Infection Control

What is the safest body mechanics principle when lifting a heavy object at the bedside?

Answer

The nurse should keep the load close to the body, bend at the knees rather than the waist, and use the large muscles of the legs to lift rather than the back. Widening the base of support also improves balance during the lift.

#117 · Safety and Infection Control

What should the nurse do immediately after a needlestick injury?

Answer

The nurse should wash the area with soap and water right away, report the incident to the supervisor, and seek assessment through occupational health or the emergency department. Post-exposure follow-up and possible prophylaxis need to be started as soon as possible for the best protection.

#118 · Safety and Infection Control

How should used needles and sharps be discarded?

Answer

Used needles and sharps must go directly into a puncture-resistant sharps container without recapping, bending, or breaking them. Recapping is one of the leading causes of needlestick injuries among healthcare workers.

#119 · Safety and Infection Control

What is the purpose of an incident report in a healthcare setting?

Answer

An incident report documents an unexpected event such as a fall, medication error, or equipment malfunction so the facility can review it, identify system problems, and prevent recurrence. It is a quality improvement tool and is kept separate from the client's health record.

#120 · Safety and Infection Control

Should details of an incident report be documented in the client's chart?

Answer

No, the incident report itself is not referenced or duplicated in the client's chart. Instead, the nurse documents the factual clinical details of what happened, the assessment findings, and the interventions given directly in the health record.

#121 · Safety and Infection Control

What are the four conditions that legally must be met before a nurse can apply a restraint?

Answer

A restraint requires a valid order or documented emergency justification, a demonstrated risk of harm to the client or others, evidence that less restrictive alternatives were tried first, and ongoing reassessment while the restraint is in place. Restraints are always a last resort, not a routine convenience measure.

#122 · Safety and Infection Control

How often should a client in physical restraints be assessed for circulation, skin integrity, and need for continued restraint?

Answer

The client should be checked at least every one to two hours, or according to facility policy, and the restraint should be released periodically for range of motion and skin care. The nurse also reassesses regularly whether the restraint is still the least restrictive option available.

#123 · Safety and Infection Control

What is the correct action if a fire is discovered on a nursing unit?

Answer

The nurse follows the RACE sequence: rescue anyone in immediate danger, activate the fire alarm, confine the fire by closing doors, then extinguish it if it is small and safe to do so, or evacuate if not. Client safety takes priority over property in every step of this sequence.

#124 · Safety and Infection Control

What does the acronym PASS describe in fire safety?

Answer

PASS describes how to use a fire extinguisher: pull the pin, aim at the base of the fire, squeeze the handle, and sweep side to side. Aiming at the base rather than the flames is what actually smothers the source of the fire.

#125 · Safety and Infection Control

How should a Workplace Hazardous Materials Information System (WHMIS) label help a nurse handle a chemical safely?

Answer

A WHMIS label identifies the hazard class of a product, required precautions, and first aid measures through standardized pictograms and text. The nurse should also consult the safety data sheet for full details on safe storage, handling, and spill response before using an unfamiliar chemical.

#126 · Safety and Infection Control

What home safety modification is most important to recommend for an older adult at risk for falls?

Answer

Removing loose throw rugs and clutter from walkways, improving lighting, and installing grab bars in the bathroom are key modifications that reduce fall risk. These changes address the most common environmental hazards found in a home fall risk assessment.

#127 · Safety and Infection Control

What should the nurse do before using any piece of medical equipment, such as an infusion pump, on a client?

Answer

The nurse should visually inspect the equipment for damage, confirm it has a current maintenance or safety inspection tag, and verify it is functioning correctly before connecting it to the client. Equipment that appears damaged or has an expired inspection should be removed from use and reported.

#128 · Safety and Infection Control

What is the correct order for donning personal protective equipment before entering an isolation room?

Answer

The order is hand hygiene, gown, mask or respirator, goggles or face shield, then gloves. Gloves go on last so they cover the cuffs of the gown and stay clean until contact with the client begins.

#129 · Safety and Infection Control

What is the correct order for removing personal protective equipment after leaving a client's room?

Answer

The order is gloves, goggles or face shield, gown, then mask or respirator, with hand hygiene performed immediately after gloves come off and again at the end. Gloves are removed first because they are the most contaminated item, and the mask is removed last since it protects the airway.

#130 · Safety and Infection Control

A client with *Clostridioides difficile* infection needs precautions. Which type applies and why?

Answer

Contact precautions apply because *C. difficile* spores spread through direct and indirect contact with contaminated surfaces or hands. Gown and gloves are required for room entry, and hands must be washed with soap and water rather than alcohol-based sanitizer since alcohol does not kill spores.

#131 · Safety and Infection Control

Which type of transmission-based precaution is used for a client with active pulmonary tuberculosis?

Answer

Airborne precautions are used because tuberculosis spreads through tiny droplet nuclei that stay suspended in air for long periods. The client needs a negative pressure room and staff must wear a fitted N95 respirator, not a surgical mask.

#132 · Safety and Infection Control

A client has influenza. What type of precaution is required and at what distance should staff maintain a mask?

Answer

Droplet precautions are required because influenza spreads through large respiratory droplets that travel short distances. A surgical mask is worn when working within about two metres of the client, and the door can remain open since the virus does not stay suspended in air.

#133 · Safety and Infection Control

What are the four main steps of standard precautions that apply to every client regardless of diagnosis?

Answer

Standard precautions include hand hygiene, use of personal protective equipment based on anticipated exposure, safe handling and disposal of sharps, and safe handling of linen and equipment contaminated with body fluids. These steps are applied to all clients because infection status is often unknown.

#134 · Safety and Infection Control

A practical nurse finds an older client sitting on the floor after a fall with no obvious injury. What is the first action?

Answer

The nurse stays with the client, checks for injury and vital signs before moving them, and calls for help rather than attempting to lift the client alone. Moving a client too quickly after a fall can worsen an undetected fracture or head injury.

#135 · Safety and Infection Control

What is a required step immediately after any client fall, even one without apparent injury?

Answer

The nurse must complete an incident report and document the event objectively in the client's chart, including what was observed and the client's response. Neurological checks and vital signs are also monitored for the next 24 to 48 hours to detect delayed complications.

#136 · Safety and Infection Control

Name three modifiable environmental risk factors for falls in a care facility.

Answer

Modifiable risk factors include poor lighting, cluttered walkways, and improperly locked wheelchairs or beds left in a raised position. Loose footwear and missing non-slip mats near bathing areas are also common contributors that staff can correct.

#137 · Safety and Infection Control

Why are bed alarms considered a restraint alternative rather than a restraint themselves?

Answer

A bed alarm does not physically restrict movement, it only alerts staff when a client attempts to get up unassisted. This allows staff to respond quickly and assist with mobility while preserving the client's freedom of movement.

#138 · Safety and Infection Control

List three least restrictive alternatives that should be tried before applying a physical restraint.

Answer

Alternatives include increasing supervision or rounding frequency, using a bed or chair alarm, and addressing the underlying cause of the behaviour such as pain, toileting needs, or confusion. Diversional activities and moving the client closer to the nursing station are also effective options.

#139 · Safety and Infection Control

Under what circumstance can a practical nurse legally apply a physical restraint without a physician's order?

Answer

A restraint can be applied without a prior order only in an emergency when the client poses an immediate risk of serious harm to self or others. A physician's order must be obtained as soon as possible afterward, and the restraint use must be documented and reassessed regularly.

#140 · Safety and Infection Control

How often must a client in physical restraints be assessed for circulation, skin integrity, and need for continued restraint?

Answer

Facility policy typically requires assessment at least every one to two hours, including checking circulation, skin condition, hydration, and toileting needs. The restraint should be released periodically during each assessment to allow movement and reduce injury risk.

#141 · Safety and Infection Control

What is the correct body mechanics principle when transferring a client from bed to wheelchair?

Answer

The nurse should keep feet shoulder-width apart for a wide base of support, bend at the knees rather than the waist, and keep the client's body close to their own during the transfer. This uses the strong leg muscles instead of the back and lowers the risk of musculoskeletal injury.

#142 · Safety and Infection Control

When should a mechanical lift, such as a Hoyer lift, be used instead of a manual two-person transfer?

Answer

A mechanical lift should be used when a client cannot bear weight, is unpredictable, or is too heavy for safe manual handling by the available staff. Many workplaces now have a no manual lift policy for any client who cannot assist with the transfer.

#143 · Safety and Infection Control

What is the purpose of a gait belt during ambulation with a weak client?

Answer

A gait belt gives the nurse a secure hand grip close to the client's centre of gravity, allowing better control if the client starts to fall. It should be applied snugly over clothing at the waist, never directly over a surgical incision or feeding tube site.

#144 · Safety and Infection Control

A client is falling while being assisted to walk. What is the safest action for the nurse to take?

Answer

The nurse should widen their stance, gently guide the client's body down the nurse's own leg to the floor rather than trying to hold the client upright or catch the full fall. This protects both the client from a hard uncontrolled fall and the nurse from back injury.

#145 · Safety and Infection Control

How should a nurse dispose of a used needle after giving an injection?

Answer

The needle should be placed uncapped directly into a puncture-resistant sharps container immediately after use, without recapping. Recapping is avoided because it is the leading cause of accidental needlestick injuries.

#146 · Safety and Infection Control

What is the first action a nurse should take immediately after a needlestick injury?

Answer

The nurse should wash the area with soap and water right away, allowing the wound to bleed briefly rather than squeezing it. The exposure must then be reported to the supervisor immediately so post-exposure risk assessment and testing can begin without delay.

#147 · Safety and Infection Control

Where should a Workplace Hazardous Materials Information System label and safety data sheet be available for a chemical used on a nursing unit?

Answer

The supplier or workplace label must stay on the container itself, and the corresponding safety data sheet must be accessible to all staff, usually through a binder or an electronic system on the unit. Staff must know how to locate this information before using any hazardous chemical.

#148 · Safety and Infection Control

What home safety hazard should a nurse specifically assess for during a home visit to an older client with a history of falls?

Answer

The nurse should check for loose throw rugs, poor lighting on stairways, absence of grab bars in the bathroom, and electrical cords running across walking paths. Uneven flooring transitions and pets that move underfoot are also common overlooked hazards in the home.

#149 · Safety and Infection Control

What information must always be included in an incident report after a medication error or equipment malfunction?

Answer

The report must include an objective, factual description of what happened, the time and location, who was involved, and the immediate actions taken, without opinions or blame. The incident report itself is not placed in the client's chart, though the clinical facts and follow up care are documented there separately.

#150 · Safety and Infection Control

What is the correct order for putting on personal protective equipment (PPE)?

Answer

The correct order is hand hygiene, gown, mask or respirator, eye protection, then gloves. Gloves go on last so they cover the gown cuffs and stay clean until contact with the patient. Following this sequence keeps the nurse protected while limiting contamination of skin and clothing.

#151 · Safety and Infection Control

What is the correct order for removing personal protective equipment?

Answer

The correct order is gloves, eye protection, gown, then mask or respirator, with hand hygiene performed immediately after gloves come off and again after all PPE is removed. Gloves are taken off first because they are the most contaminated item. The mask or respirator is removed last and by the ties or ear loops only, never by touching the front.

#152 · Safety and Infection Control

A practical nurse notices a coworker gave the wrong dose of insulin but the patient had no ill effects. Should an incident report still be filed?

Answer

Yes, an incident report must be filed any time an error occurs, even if the patient shows no harm. The report documents the facts objectively, supports patient monitoring, and helps the organization identify system problems so the error does not repeat. It is a quality and safety tool, not a punishment record.

#153 · Safety and Infection Control

Should a copy of an incident report be placed in the patient's health record?

Answer

No, the incident report itself is never filed in the patient's chart because it is an internal risk management document. The nurse still documents the facts of the event, the assessment findings, and the interventions given directly in the patient's health record. The two documents stay separate but both must be completed.

#154 · Safety and Infection Control

What four pieces of information does a WHMIS label always show?

Answer

A WHMIS supplier label always shows the product identifier, a pictogram showing the type of hazard, a signal word such as danger or warning, and hazard statements describing the risk. It also includes precautionary statements and supplier information. This label is the worker's first quick source of hazard information before consulting the safety data sheet.

#155 · Safety and Infection Control

Where can a worker find detailed first aid and spill cleanup instructions for a hazardous chemical used on a unit?

Answer

This information is found on the safety data sheet, formerly called the material safety data sheet, kept on file for every hazardous product on the unit. The safety data sheet lists sixteen standardized sections including first aid measures, handling, storage, and disposal. Every worker who handles the product must know how to access this sheet.

#156 · Safety and Infection Control

What are the three parts of the WHMIS 2015 information system that every employer must provide to workers?

Answer

The three parts are supplier labels on hazardous products, safety data sheets for each hazardous product, and worker education and training on how to use both. Employers must also provide workplace labels when a product is transferred out of its original container. Together these parts let a worker recognize a hazard and respond safely.

#157 · Safety and Infection Control

During a fire on a nursing unit, what does the acronym RACE stand for?

Answer

RACE stands for rescue anyone in immediate danger, activate the alarm, confine the fire by closing doors and windows, and extinguish the fire if it is small and safe to do so or evacuate if it is not. This sequence keeps the priority on protecting people before property. Staff should know their unit's fire plan and the location of pull stations and extinguishers.

#158 · Safety and Infection Control

What does the acronym PASS describe when using a fire extinguisher?

Answer

PASS stands for pull the pin, aim the nozzle at the base of the fire, squeeze the handle, and sweep side to side across the base of the flames. Aiming at the base rather than the top of the flames is what actually smothers the fuel source. A nurse should only attempt this on a small, contained fire.

#159 · Safety and Infection Control

In a hospital's color coded emergency code system, what does code red typically mean?

Answer

Code red typically signals a fire or smoke emergency somewhere in the facility. Staff respond according to the unit fire and evacuation plan, which usually follows the RACE sequence. Exact code meanings can vary slightly by province and facility, so every nurse should confirm the local code policy on orientation.

#160 · Safety and Infection Control

What is the single most important principle when lifting or transferring a patient manually?

Answer

The nurse keeps the load close to the body, bends at the knees rather than the waist, and uses leg muscles instead of back muscles to lift. A wide base of support and facing the direction of movement also protect the spine. Mechanical lifts should be used whenever the patient cannot bear weight or the lift is beyond safe limits.

#161 · Safety and Infection Control

Why is a mechanical or ceiling lift preferred over a manual two person transfer for a patient who cannot bear any weight?

Answer

A mechanical lift removes most of the physical strain from the caregiver's spine and shoulders, which lowers the risk of a musculoskeletal injury. It also gives the patient a more stable, controlled transfer with less risk of a fall or skin shear. Many workplace safety policies now require a mechanical lift whenever a patient scores as a total dependent transfer.

#162 · Safety and Infection Control

What is the first action a nurse takes immediately after a needlestick injury from a used sharp?

Answer

The nurse washes the site right away with soap and water, allowing it to bleed gently rather than squeezing it. The injury is then reported to the supervisor immediately so post exposure testing and prophylaxis can be arranged without delay. An incident report and occupational health follow up are also completed.

#163 · Safety and Infection Control

What is the correct way to dispose of a used needle after an injection?

Answer

The needle is placed uncapped directly into a puncture resistant sharps container immediately after use, without recapping, bending, or breaking it. Recapping is the leading cause of preventable needlestick injuries. The sharps container should be within arm's reach at the point of care and never overfilled past the fill line.

#164 · Safety and Infection Control

Which type of precautions applies to a patient with active pulmonary tuberculosis?

Answer

Airborne precautions apply, which require a negative pressure room and an N95 respirator or equivalent fit tested mask for anyone entering. The door must stay closed to maintain the pressure differential. Other airborne pathogens include measles and varicella, though standard precautions are still used for every patient regardless of diagnosis.

#165 · Safety and Infection Control

What type of precautions and PPE are needed for a patient with influenza?

Answer

Droplet precautions are used, which require a surgical or procedure mask when working within about two metres of the patient, along with standard precautions such as hand hygiene and gloves. Eye protection is added if splashing is likely. The patient's door does not need to stay closed the way it does for airborne precautions.

#166 · Safety and Infection Control

What type of precautions applies to a patient colonized with methicillin resistant *Staphylococcus aureus* (MRSA)?

Answer

Contact precautions apply, requiring a gown and gloves for any direct contact with the patient or their environment. Equipment used for the patient should be dedicated to that room whenever possible or cleaned thoroughly before use on another patient. Hand hygiene remains essential before and after every contact even with gloves worn.

#167 · Safety and Infection Control

Under Canadian restraint policy, what must a nurse assess and document before a physical restraint is applied?

Answer

The nurse must first try and document less restrictive alternatives, then confirm a physician or authorized order exists that specifies the type and reason for the restraint. Consent from the patient or substitute decision maker is required except in a true emergency. The restraint is used only as a last resort to prevent immediate harm, never for staff convenience.

#168 · Safety and Infection Control

How often must a patient in physical restraints be assessed for circulation, skin integrity, and continued need for the restraint?

Answer

Facility policies vary but generally require assessment at least every one to two hours, with circulation and skin checked and the restraint released briefly to allow range of motion. The ongoing need for the restraint must be reassessed at each check rather than assumed to still apply. All findings and the rationale for continuing the restraint are documented each time.

#169 · Safety and Infection Control

What home safety modification most reduces fall risk for an older adult with impaired mobility?

Answer

Removing loose rugs and clutter from walkways and adding grab bars in the bathroom are among the most effective changes because most falls happen during transfers and on unsecured floor surfaces. Adequate lighting, especially at night, and non slip mats in the tub also lower risk. A home safety assessment should be individualized to the specific hazards found in that person's living space.

#170 · Safety and Infection Control

Before using any piece of medical equipment such as an infusion pump, what safety check must the nurse perform?

Answer

The nurse checks that the equipment has a current inspection or maintenance tag, inspects the cord and casing for visible damage, and confirms it is functioning correctly before connecting it to a patient. Any equipment that appears damaged or overdue for inspection is removed from use and reported through the facility's equipment reporting process. This check prevents electrical hazards and inaccurate delivery of fluids or medications.

#171 · Health Promotion and Maintenance

What is the normal heart rate range for a healthy newborn?

Answer

A healthy newborn's heart rate normally ranges from 110 to 160 beats per minute, though it can briefly rise to 180 during crying or drop to 100 during deep sleep. The practical nurse should count the apical pulse for a full minute to get an accurate reading.

#172 · Health Promotion and Maintenance

At what age does a typical infant develop a pincer grasp?

Answer

The pincer grasp, in which the infant picks up small objects using the thumb and index finger, typically develops between 9 and 12 months of age. This milestone signals maturing fine motor control and readiness for self-feeding with small pieces of food.

#173 · Health Promotion and Maintenance

What is the recommended timing for administering vitamin K to a newborn?

Answer

Vitamin K is given as a single intramuscular injection into the vastus lateralis muscle within the first 1 to 2 hours after birth. It prevents vitamin K deficiency bleeding because newborns are born with low levels of vitamin K and sterile bowels that cannot yet synthesize it.

#174 · Health Promotion and Maintenance

What does a positive Ortolani sign indicate in a newborn assessment?

Answer

A positive Ortolani sign indicates developmental dysplasia of the hip, felt as a clunk when the examiner abducts the newborn's flexed hips and the femoral head slips back into the acetabulum. Early identification allows timely referral and treatment with a Pavlik harness to promote normal hip joint development.

#175 · Health Promotion and Maintenance

What is the expected fundal height in centimeters at 20 weeks gestation?

Answer

At 20 weeks gestation, the fundal height measured in centimeters from the symphysis pubis to the top of the uterus should approximately equal the number of weeks of gestation, so about 20 centimeters. This McDonald's rule estimate is used between 20 and 34 weeks to screen for appropriate fetal growth.

#176 · Health Promotion and Maintenance

What danger sign during pregnancy should a client report immediately?

Answer

A client should immediately report vaginal bleeding, severe or persistent headache, visual disturbances, sudden swelling of the face or hands, decreased fetal movement, or leaking amniotic fluid. These signs can indicate complications such as placental abruption, preeclampsia, or fetal compromise that require urgent assessment.

#177 · Health Promotion and Maintenance

What is the normal range for amniotic fluid volume assessed by amniotic fluid index?

Answer

A normal amniotic fluid index falls between 5 and 25 centimeters. Values below 5 centimeters indicate oligohydramnios and values above 25 centimeters indicate polyhydramnios, both of which require further fetal and maternal evaluation.

#178 · Health Promotion and Maintenance

During the postpartum period, where should the fundus be located immediately after delivery of the placenta?

Answer

Immediately after the placenta is delivered, the fundus should be firm and located at or slightly below the level of the umbilicus, at the midline. The nurse should assess fundal height and firmness every 15 minutes during the first hour to detect early signs of postpartum hemorrhage.

#179 · Health Promotion and Maintenance

How quickly does the postpartum fundus normally descend each day after birth?

Answer

The fundus descends approximately 1 to 2 centimeters, or roughly one fingerbreadth, each day after birth. By about 10 to 14 days postpartum the uterus should no longer be palpable above the symphysis pubis as it returns to its pre-pregnancy size.

#180 · Health Promotion and Maintenance

What lochia pattern is expected in the first 3 days after vaginal delivery?

Answer

In the first 3 days after delivery, lochia rubra is expected, consisting of a dark red discharge made up mainly of blood, decidual tissue, and mucus. Saturating more than one pad per hour or passing large clots is abnormal and should be reported right away.

#181 · Health Promotion and Maintenance

What is the correct sequence of lochia color changes across the postpartum period?

Answer

Lochia progresses from lochia rubra, a dark red discharge in days 1 to 3, to lochia serosa, a pinkish brown discharge from about days 4 to 10, and finally to lochia alba, a yellowish white discharge that can persist until about 6 weeks postpartum. A reversal back to rubra after serosa has started may signal overexertion or delayed healing.

#182 · Health Promotion and Maintenance

What newborn reflex is tested by stroking the sole of the foot from heel to toe?

Answer

Stroking the sole from heel toward the toes tests the Babinski reflex, and a positive response is fanning and hyperextension of the toes. This reflex is a normal finding in newborns and typically disappears by about 12 months of age as the nervous system matures.

#183 · Health Promotion and Maintenance

At what age does the Moro reflex normally disappear?

Answer

The Moro, or startle, reflex normally disappears by 4 to 6 months of age. Persistence of this reflex beyond 6 months can be a sign of a neurological problem and should be reported for further evaluation.

#184 · Health Promotion and Maintenance

What should the nurse teach parents about safe infant sleep positioning?

Answer

Parents should be taught to place healthy infants on their backs to sleep for every sleep, on a firm flat surface free of pillows, blankets, bumper pads, and soft toys. This back-to-sleep practice significantly reduces the risk of sudden infant death syndrome.

#185 · Health Promotion and Maintenance

According to Erikson's theory, what psychosocial task is the infant, from birth to 1 year, working to resolve?

Answer

According to Erikson, the infant is working through trust versus mistrust during the first year of life. Consistent, responsive caregiving that meets the infant's needs for food, comfort, and warmth builds a sense of trust in the world.

#186 · Health Promotion and Maintenance

According to Erikson's theory, what psychosocial task does the toddler, roughly ages 1 to 3, work to resolve?

Answer

The toddler works through autonomy versus shame and doubt, developing independence through activities like self-feeding, toilet learning, and making simple choices. Caregivers support this stage by allowing safe opportunities for the toddler to try tasks independently rather than doing everything for the child.

#187 · Health Promotion and Maintenance

By what age should an infant typically be able to sit unsupported?

Answer

Most infants can sit unsupported by 6 to 8 months of age, after first developing the head and trunk control needed to maintain balance in this position. This milestone usually follows rolling over and precedes crawling.

#188 · Health Promotion and Maintenance

According to Canadian immunization schedules, at what ages is the routine primary series of DTaP-IPV-Hib typically given?

Answer

The primary series is typically given at 2, 4, and 6 months of age, with a booster dose in the toddler or preschool years depending on the provincial schedule. Ontario and British Columbia both follow this general 2, 4, 6 month primary series pattern for combined diphtheria, tetanus, pertussis, polio, and Haemophilus influenzae type b vaccination.

#189 · Health Promotion and Maintenance

What is the recommended exclusive breastfeeding duration according to Health Canada?

Answer

Health Canada recommends exclusive breastfeeding for the first 6 months of life, with breastfeeding continuing alongside appropriate complementary foods for up to 2 years or longer. Exclusive breastfeeding means the infant receives only breast milk, with no other liquids or solid foods except vitamin D supplementation.

#190 · Health Promotion and Maintenance

Why is a daily vitamin D supplement recommended for exclusively breastfed infants in Canada?

Answer

Breast milk alone does not provide enough vitamin D to meet an infant's needs, so Health Canada recommends a daily supplement of 400 international units for exclusively or partially breastfed infants. Adequate vitamin D intake helps prevent rickets and supports healthy bone development.

#191 · Health Promotion and Maintenance

What barrier contraceptive method also offers protection against sexually transmitted infections?

Answer

The male condom is a barrier method that both prevents pregnancy and reduces the risk of transmitting sexually transmitted infections when used correctly and consistently. It is often recommended alongside another method for clients wanting both dual protection and higher pregnancy prevention effectiveness.

#192 · Health Promotion and Maintenance

What is the purpose of the Pap test in a routine health screening visit?

Answer

The Pap test screens for abnormal cervical cell changes that can indicate precancerous conditions or cervical cancer, often caused by persistent infection with high risk strains of human papillomavirus. Early detection through regular screening allows treatment before abnormal cells progress to invasive cancer.

#193 · Health Promotion and Maintenance

What physical assessment finding in an older adult is considered a normal age-related change rather than a sign of illness?

Answer

A decrease in skin turgor with thinner, drier, more wrinkled skin is a normal age-related change caused by decreased collagen and subcutaneous fat, not a sign of dehydration in an older adult by itself. The nurse should use other indicators, such as mucous membrane moisture and urine output, to properly assess hydration status in this population.

#194 · Health Promotion and Maintenance

What are the three stages of labour, and what marks the start and end of each?

Answer

Stage one runs from the onset of true labour contractions to full cervical dilation of 10 cm and is divided into latent, active, and transition phases. Stage two goes from full dilation to the birth of the baby. Stage three begins after the baby is born and ends when the placenta is delivered.

#195 · Health Promotion and Maintenance

A postpartum client's fundus is boggy and displaced above and to the right of the umbilicus. What should the nurse suspect and do first?

Answer

This pattern suggests a full bladder is preventing the uterus from contracting properly, which raises the risk of postpartum hemorrhage. The nurse should have the client void or catheterize if needed, then reassess and massage the fundus until it is firm.

#196 · Health Promotion and Maintenance

What is the normal range for a newborn's Apgar score at 5 minutes, and what does a low score indicate?

Answer

A 5 minute Apgar score of 7 to 10 is considered normal and reflects good adjustment to extrauterine life. A score below 7 indicates the newborn is having difficulty transitioning and needs continued assessment or resuscitative support.

#197 · Health Promotion and Maintenance

What is the difference between lochia rubra, serosa, and alba, and when does each normally occur?

Answer

Lochia rubra is red and occurs for the first 3 to 4 days postpartum as mostly blood. Lochia serosa is pinkish brown and appears from about day 4 to day 10 as blood mixes with serous fluid. Lochia alba is yellowish white and can last from day 10 up to 6 weeks postpartum as healing continues.

#198 · Health Promotion and Maintenance

A client is 6 hours postpartum and reports severe perineal pain with a feeling of pressure in the rectum. What complication should the nurse assess for?

Answer

These symptoms suggest a perineal or vulvar hematoma from trauma during birth. The nurse should inspect the perineum for swelling and discoloration, apply ice if an early hematoma is found, and notify the care provider promptly since a large hematoma can cause significant blood loss.

#199 · Health Promotion and Maintenance

What are the classic signs of a developing deep vein thrombosis in a postpartum client?

Answer

Signs include unilateral leg swelling, warmth, redness, and calf pain or tenderness, sometimes with a palpable cord along the vein. The nurse should avoid massaging the area and should notify the care provider immediately because of the risk of pulmonary embolism.

#200 · Health Promotion and Maintenance

What laboratory and physical findings define preeclampsia in pregnancy?

Answer

Preeclampsia is defined by a blood pressure of 140/90 mmHg or higher after 20 weeks gestation in a previously normotensive client, along with proteinuria or other signs of organ dysfunction such as low platelets, elevated liver enzymes, or headache and visual changes. It requires close monitoring because it can progress to eclampsia with seizures.

#201 · Health Promotion and Maintenance

Why is magnesium sulfate given to clients with severe preeclampsia, and what is the first sign of toxicity the nurse should watch for?

Answer

Magnesium sulfate is given to prevent eclamptic seizures by depressing the central nervous system. The earliest sign of toxicity is usually loss of deep tendon reflexes, so the nurse checks reflexes along with respiratory rate and urine output regularly during the infusion.

#202 · Health Promotion and Maintenance

What is the recommended timing and technique for initiating skin to skin contact after an uncomplicated vaginal birth?

Answer

Skin to skin contact should begin immediately after birth and continue uninterrupted for at least the first hour when both the newborn and client are stable. The dried newborn is placed prone on the client's bare chest and covered with a warm blanket to support thermoregulation, bonding, and early breastfeeding initiation.

#203 · Health Promotion and Maintenance

A newborn develops yellowing of the skin within the first 24 hours of life. Why is this considered concerning rather than normal?

Answer

Jaundice appearing in the first 24 hours is called pathological jaundice and is not explained by the usual breakdown of fetal red blood cells seen in physiological jaundice. It often points to a cause such as blood type incompatibility or infection and needs prompt bilirubin testing and follow up.

#204 · Health Promotion and Maintenance

What are the four stages of Jean Piaget's cognitive development theory, in order?

Answer

The stages are sensorimotor from birth to about 2 years, preoperational from about 2 to 7 years, concrete operational from about 7 to 11 years, and formal operational from about 11 years onward. Each stage reflects a different way that children reason about and understand the world around them.

#205 · Health Promotion and Maintenance

According to Erikson's stages of psychosocial development, what is the main developmental task of older adulthood?

Answer

Erikson describes the task of older adulthood as ego integrity versus despair. A person who looks back on life with a sense of satisfaction and meaning achieves integrity, while someone who feels regret over unmet goals may experience despair.

#206 · Health Promotion and Maintenance

What physiological changes in the aging heart and blood vessels increase the risk of orthostatic hypotension in older adults?

Answer

Aging arteries lose elasticity and stiffen, and the baroreceptor reflex that normally adjusts heart rate and vessel tone in response to position changes becomes slower and less sensitive. Together these changes mean blood pressure drops more when an older adult stands up and takes longer to recover, raising the risk of dizziness and falls.

#207 · Health Promotion and Maintenance

What are the recommended screening intervals for colorectal cancer in average risk adults in Canada?

Answer

Average risk adults aged 50 to 74 are generally advised to have a fecal immunochemical test every 1 to 2 years, or a colonoscopy every 10 years if that method is chosen instead. Screening may start earlier or occur more often for people with a family history or other risk factors.

#208 · Health Promotion and Maintenance

Why are older adults at higher risk for adverse drug reactions even at standard doses?

Answer

Aging reduces liver and kidney function, which slows drug metabolism and clearance, and changes in body composition alter how drugs distribute in the body. Combined with polypharmacy from multiple chronic conditions, these changes mean standard doses can build up to toxic levels more easily than in younger adults.

#209 · Health Promotion and Maintenance

What is the difference between delirium and dementia in an older adult, and why does this distinction matter for nursing care?

Answer

Delirium has a sudden onset, fluctuates through the day, and is often caused by a reversible problem such as infection or medication effect, while dementia develops gradually and progresses over months to years without full recovery. Recognizing sudden confusion as possible delirium is important because prompt treatment of the underlying cause can reverse it, whereas dementia cannot be cured.

#210 · Health Promotion and Maintenance

What are the two main types of hormonal contraceptive methods and one key difference in how they work?

Answer

Combined hormonal methods contain both estrogen and progestin and primarily work by preventing ovulation. Progestin only methods work mainly by thickening cervical mucus and thinning the uterine lining, and they are often chosen for clients who cannot take estrogen, such as those who are breastfeeding or have a history of blood clots.

#211 · Health Promotion and Maintenance

What are the routine components of a well baby physical assessment in the first days of life?

Answer

The assessment includes measuring weight, length, and head circumference, checking vital signs, and inspecting the skin, fontanelles, reflexes such as rooting and grasp, and cardiac and respiratory function. The nurse also checks for hip stability, patent anus, and normal voiding and stooling patterns before discharge.

#212 · Health Promotion and Maintenance

What is the recommended immunization schedule pattern for the routine childhood series in Canada, and why are multiple doses needed?

Answer

Most routine vaccines are given in a series starting at 2 months of age with additional doses at set intervals through early childhood, such as at 4, 6, and 12 to 18 months. Multiple doses are needed because a young immune system often does not build strong enough or long enough protection from a single dose alone.

#213 · Health Promotion and Maintenance

A client asks why the nurse recommends waiting until 4 to 6 weeks postpartum before resuming sexual intercourse. What is the clinical reasoning?

Answer

This waiting period allows the cervix to close, the uterine lining to heal from where the placenta detached, and any perineal or cesarean incision to recover, which lowers the risk of infection and bleeding. It also gives time for lochia to resolve, since intercourse before healing is complete increases the chance of introducing bacteria into the uterus.

#214 · Health Promotion and Maintenance

What findings suggest a client may be developing postpartum depression rather than the common baby blues?

Answer

Baby blues typically involve mild mood swings, tearfulness, and irritability that peak around day 3 to 5 and resolve within about 2 weeks without treatment. Postpartum depression involves persistent sadness, loss of interest, feelings of worthlessness, or difficulty bonding with the baby that last longer than 2 weeks and interfere with daily functioning, and it needs professional evaluation and treatment.

#215 · Health Promotion and Maintenance

What lifestyle counselling should the nurse provide about bone health and fall prevention for a postmenopausal client?

Answer

The nurse should encourage adequate calcium and vitamin D intake, regular weight bearing exercise, and smoking cessation to help maintain bone density after the drop in estrogen that occurs with menopause. The nurse should also address home safety, footwear, and vision checks since falls in someone with lower bone density carry a higher risk of fracture.

#216 · Psychosocial Integrity

What therapeutic communication technique involves repeating part of what a client just said to encourage them to continue talking?

Answer

This is called restating. The nurse repeats a key word or phrase back to the client in a neutral tone, which shows the nurse is listening and invites the client to expand on that point without the nurse adding interpretation.

#217 · Psychosocial Integrity

During a crisis intervention, what is the first priority before attempting any de-escalation technique?

Answer

Safety of the client, the nurse, and everyone nearby comes first. The nurse assesses for weapons, exit routes, and escalating body language before engaging verbally, and calls for backup if the situation looks unsafe.

#218 · Psychosocial Integrity

A practical nurse suspects a child in their care is being abused at home. What is the nurse legally required to do in most Canadian provinces?

Answer

The nurse must report the suspicion directly to the local child protection agency, such as a Children's Aid Society in Ontario. This duty applies even on reasonable suspicion, not proof, and overrides normal confidentiality rules.

#219 · Psychosocial Integrity

What is the difference between sympathy and empathy in a therapeutic nurse-client relationship?

Answer

Sympathy means feeling sorry for a client from the nurse's own perspective, which can create distance or pity. Empathy means understanding and reflecting the client's feelings from their perspective while staying objective, which builds trust and keeps the relationship therapeutic.

#220 · Psychosocial Integrity

A client with schizophrenia tells the nurse, 'The voices are telling me to hurt myself.' What should the nurse say first?

Answer

The nurse should acknowledge the client's experience without arguing about the reality of the voices and ask directly about safety, such as 'What are the voices telling you to do, and do you have a plan to hurt yourself right now?' Direct safety questions never increase suicide risk and are essential for triage.

#221 · Psychosocial Integrity

What are the four stages of Kubler-Ross's grief model that most commonly appear on Canadian nursing exams besides denial and anger?

Answer

The remaining stages are bargaining, depression, and acceptance. Nurses should remember clients do not move through these stages in a fixed order and may move back and forth or skip stages entirely.

#222 · Psychosocial Integrity

What is the correct nursing action when a family member asks the nurse not to tell a dying client their diagnosis?

Answer

The nurse should explore the family's concerns and explain that the client has the right to know their own health information unless the client has clearly stated they do not want to know. The nurse then facilitates a discussion between the family, client, and physician rather than agreeing to withhold information unilaterally.

#223 · Psychosocial Integrity

A client who used to drink daily suddenly stops all alcohol intake after being admitted to hospital. What is the priority nursing assessment over the next 24 to 72 hours?

Answer

The nurse monitors closely for alcohol withdrawal syndrome, including tremors, elevated heart rate and blood pressure, sweating, agitation, and the risk of seizures or delirium tremens. Vital signs and a withdrawal scale such as CIWA-Ar should be used on a regular schedule.

#224 · Psychosocial Integrity

What communication technique should the nurse avoid because it shifts focus away from the client's feelings and onto the nurse's own experience?

Answer

This is giving personal opinions or self-disclosure that is not client-centred, such as saying 'I know exactly how you feel because the same thing happened to me.' It can minimize the client's unique experience and redirect attention away from them.

#225 · Psychosocial Integrity

What is the term for a nurse's unconscious emotional reaction to a client based on the nurse's own past relationships?

Answer

This is countertransference. It happens when the nurse projects feelings from a past relationship onto the client, which can cloud objectivity, so the nurse should recognize it and seek supervision or peer support to manage it.

#226 · Psychosocial Integrity

A client with borderline personality disorder frequently calls the nurse 'the only one who understands me' one day and 'useless' the next. What defense mechanism does this pattern reflect?

Answer

This pattern is called splitting. The client views people as all good or all bad with no middle ground, and the nursing team should maintain consistent boundaries and communicate as a team to avoid being manipulated by the split.

#227 · Psychosocial Integrity

What is the recommended physical stance a nurse should take when de-escalating an agitated client?

Answer

The nurse should stand at an angle rather than directly facing the client, keep at least an arm's length of distance, and avoid crossing arms or pointing. This posture appears less confrontational and keeps an exit route open for both people.

#228 · Psychosocial Integrity

What is the difference between neglect and abandonment when assessing an older adult for elder mistreatment?

Answer

Neglect is the failure to provide necessary care such as food, hygiene, or medical attention, whether intentional or from caregiver burnout. Abandonment is the desertion of a dependent older adult by the person responsible for their care, leaving them entirely without support.

#229 · Psychosocial Integrity

A client says, 'I feel like nobody cares about me anymore.' What is the best therapeutic response?

Answer

The nurse should use reflection, such as 'You're feeling alone right now.' This validates the client's emotion and invites them to say more, rather than offering false reassurance like 'Of course people care about you,' which shuts down the conversation.

#230 · Psychosocial Integrity

What are the three phases of Peplau's nurse-client relationship model?

Answer

The three phases are orientation, working, and termination. In orientation the nurse and client build trust and set goals, in the working phase they address the client's problems, and in termination they review progress and prepare to end the relationship.

#231 · Psychosocial Integrity

A client from a culture that values indirect eye contact as a sign of respect avoids looking at the nurse during conversation. How should the nurse interpret this?

Answer

The nurse should recognize this as a culturally appropriate behaviour rather than a sign of dishonesty or disengagement. Cultural norms around eye contact vary widely, and the nurse should adapt their own expectations instead of assuming the client is being evasive.

#232 · Psychosocial Integrity

What is the term for a coping strategy that directly addresses the source of stress, such as making a study plan before an exam?

Answer

This is problem-focused coping. It contrasts with emotion-focused coping, which manages the feelings caused by stress, such as relaxation techniques, without changing the actual stressor.

#233 · Psychosocial Integrity

A client in acute crisis is talking rapidly, pacing, and unable to focus on the nurse's questions. What is the priority nursing intervention?

Answer

The nurse should use a calm, low, and slow tone of voice, give simple one-step instructions, and reduce environmental stimulation such as noise and bright lights. Matching the client's energy or rushing them will usually increase agitation rather than help.

#234 · Psychosocial Integrity

What is the difference between open-ended and closed-ended questions in therapeutic communication, and when should each be used?

Answer

Open-ended questions invite a detailed response, such as 'How have you been feeling this week,' and are best for exploring feelings. Closed-ended questions have a short or yes-or-no answer, such as 'Did you take your medication today,' and are best for gathering specific factual information quickly.

#235 · Psychosocial Integrity

A family member of a critically ill client keeps asking the nurse detailed medical questions instead of visiting the bedside. What underlying need might this behaviour reflect?

Answer

This behaviour often reflects the family member's need for a sense of control and information during a frightening and uncertain situation. The nurse should answer honestly within their scope, validate the family's anxiety, and gently encourage bedside presence when appropriate.

#236 · Psychosocial Integrity

What sensory alteration is most common in older adults and can be mistaken for confusion or dementia if not identified?

Answer

Age-related hearing loss, called presbycusis, is very common and can cause an older adult to seem confused or unresponsive simply because they did not hear the question. The nurse should check hearing aids, face the client when speaking, and reduce background noise before assuming a cognitive problem.

#237 · Psychosocial Integrity

What is the nursing priority when a client discloses they are in an abusive relationship but insists on returning home to their partner?

Answer

The nurse respects the client's autonomy to make their own decision while providing information on safety planning, local shelters, and crisis line numbers. The nurse should avoid pressuring the client to leave, since this can damage trust and reduce the chance the client will reach out for help later.

#238 · Psychosocial Integrity

What is the term for using silence purposefully during a therapeutic conversation?

Answer

This is therapeutic use of silence. Pausing gives the client time to process their thoughts and feelings and often encourages them to share more without being interrupted or prompted.

#239 · Psychosocial Integrity

A client newly diagnosed with a terminal illness says, 'If I can just make it to my daughter's wedding, I'll accept whatever happens after that.' Which stage of grief does this reflect?

Answer

This reflects the bargaining stage of grief. The client is attempting to negotiate more time or a better outcome, often with a higher power or fate, in exchange for accepting the illness.

#240 · Psychosocial Integrity

What defines a support system in the context of psychosocial nursing assessment?

Answer

A support system includes the people, groups, and resources a client can rely on for emotional, physical, or practical help, such as family, friends, faith communities, or community agencies. The nurse assesses the strength and availability of this network because it directly affects the client's ability to cope and recover.

#241 · Psychosocial Integrity

What is the correct nursing response when a colleague asks about a client's psychiatric history in the hallway out of curiosity, not for care purposes?

Answer

The nurse should decline to share the information, since access to health records is limited to those with a legitimate need to know for that client's care. This protects the client's right to confidentiality under provincial privacy legislation.

#242 · Psychosocial Integrity

A client with major depressive disorder who was withdrawn for weeks suddenly seems calm, energetic, and starts giving away personal belongings. What should the nurse suspect?

Answer

The nurse should suspect the client may have decided on a plan to end their life, since a sudden lift in mood after a period of severe depression can signal relief that a suicide decision has been made. The nurse should immediately increase observation and complete a thorough suicide risk assessment.

#243 · Psychosocial Integrity

What is the key difference between clarifying and paraphrasing as therapeutic communication techniques?

Answer

Clarifying is asking the client to explain something further because the nurse did not fully understand, such as 'Can you tell me more about what you mean by that.' Paraphrasing is restating the client's message in the nurse's own words to confirm understanding, such as 'It sounds like you're saying the pain gets worse at night.'

#244 · Psychosocial Integrity

What are the earliest signs of alcohol withdrawal and when do they typically start after the last drink?

Answer

Early signs include tremor, anxiety, sweating, nausea, insomnia, and mild tachycardia. These usually begin within 6 to 12 hours after the last drink and peak around 24 to 48 hours. The practical nurse should monitor vital signs closely during this window because untreated withdrawal can progress to seizures or delirium tremens.

#245 · Psychosocial Integrity

What is delirium tremens and why is it a medical emergency?

Answer

Delirium tremens is a severe form of alcohol withdrawal that usually appears 48 to 96 hours after the last drink, marked by severe confusion, hallucinations, autonomic instability, and seizures. It carries a real risk of death from cardiovascular collapse or uncontrolled seizure activity if untreated. Nurses should report new confusion or fever in a withdrawing client immediately rather than waiting for the next scheduled assessment.

#246 · Psychosocial Integrity

Which assessment tool do Canadian nurses commonly use to score the severity of alcohol withdrawal?

Answer

The Clinical Institute Withdrawal Assessment for Alcohol, revised version, known as CIWA-Ar, is the standard tool. It scores symptoms such as tremor, sweating, anxiety, agitation, and hallucinations to guide as-needed benzodiazepine dosing. Frequent reassessment using this scale allows the care team to titrate medication safely instead of using a fixed dosing schedule.

#247 · Psychosocial Integrity

Why are benzodiazepines the first-line medication class for treating alcohol withdrawal?

Answer

Benzodiazepines act on the same gamma-aminobutyric acid receptors that alcohol affects, so they calm the overactive central nervous system and reduce seizure risk during withdrawal. Common choices include diazepam and lorazepam, with lorazepam often preferred in clients with liver impairment because it does not depend on hepatic metabolism to the same degree. The nurse should still monitor for oversedation and respiratory depression during treatment.

#248 · Psychosocial Integrity

What is the correct nursing action if opioid withdrawal is suspected in a hospitalized client?

Answer

The nurse should assess for signs such as yawning, rhinorrhea, piloerection, muscle aches, abdominal cramping, and dilated pupils, then notify the prescriber for symptom management rather than assume the client is simply drug seeking. Comfort measures and prescribed medications such as clonidine or methadone can reduce symptoms while keeping the client safe. Judgmental attitudes toward the client can damage trust and discourage honest reporting of symptoms.

#249 · Psychosocial Integrity

Is opioid withdrawal typically life-threatening compared with alcohol or benzodiazepine withdrawal?

Answer

Opioid withdrawal causes intense physical discomfort but is rarely fatal on its own in an otherwise healthy adult. In contrast, alcohol and benzodiazepine withdrawal can cause seizures and death if untreated because of central nervous system hyperexcitability. This distinction affects triage priority when a client reports substance use in multiple categories.

#250 · Psychosocial Integrity

What is harm reduction in the context of substance use care?

Answer

Harm reduction is an approach that aims to reduce the negative consequences of substance use without requiring abstinence as a condition of care. Examples include needle exchange programs, supervised consumption sites, and naloxone distribution. A practical nurse supports harm reduction by providing nonjudgmental care and teaching safer use practices even if the client is not ready to stop using.

#251 · Psychosocial Integrity

What should a nurse teach a client and family about naloxone before discharge after an opioid-related event?

Answer

The nurse should explain that naloxone temporarily reverses opioid overdose by blocking opioid receptors, but its effects wear off before many opioids do, so repeat dosing and emergency services are still needed. Family members should be shown how to recognize an overdose and how to administer the nasal spray or injectable form. The client should also be told that naloxone can trigger sudden withdrawal symptoms once it takes effect.

#252 · Psychosocial Integrity

Under Canadian provincial child welfare legislation, who is required to report suspected child abuse or neglect?

Answer

Every person, including nurses, is legally required to report a suspicion of child abuse or neglect directly to the local child welfare authority, without waiting for proof or for a supervisor's approval. This duty applies even when the nurse has been told the information in confidence. Failing to report can result in legal penalties for the nurse.

#253 · Psychosocial Integrity

A nurse suspects an elderly client in a long-term care facility is being financially and physically abused by a family caregiver. What is the priority action?

Answer

The nurse should document objective findings such as bruising patterns and behavioural changes, then report the suspicion through the facility's protection protocol and to the appropriate provincial adult protection or regulatory body. The client's immediate safety takes priority, so the nurse should also ensure the client is not left alone with the suspected abuser during the assessment. Reporting is required even without absolute proof of abuse.

#254 · Psychosocial Integrity

What physical findings should raise suspicion of nonaccidental injury in a young child rather than an accidental one?

Answer

Bruises in unusual locations such as the torso, ears, or genitals, injuries in different stages of healing, and a history that does not match the child's developmental ability are all red flags. Spiral fractures in a nonambulatory infant are also highly suspicious. The nurse must document exact location, size, and appearance objectively without stating a conclusion about how the injury occurred.

#255 · Psychosocial Integrity

How should a nurse document a disclosure of abuse made by a client?

Answer

The nurse should record the client's exact words in quotation marks rather than paraphrasing, along with the date, time, and who was present. Objective physical findings should be described factually, such as size and location of bruising, without adding personal interpretation. This accurate documentation may later be used in legal proceedings.

#256 · Psychosocial Integrity

What are the five stages of grief described by Kubler-Ross, and how should a nurse use this framework?

Answer

The stages are denial, anger, bargaining, depression, and acceptance. The nurse should understand that clients and families do not move through these stages in a fixed order and may revisit earlier stages, so the framework is a guide to understanding reactions rather than a checklist to complete. The nurse's role is to support the person wherever they are in the process, not to push them toward acceptance.

#257 · Psychosocial Integrity

What is anticipatory grief and how does it differ from grief after a death has occurred?

Answer

Anticipatory grief is mourning that begins before an expected loss, such as when a family learns a diagnosis is terminal. It allows family members time to begin adjusting emotionally and practically, but it does not eliminate the grief reaction after the death actually occurs. Nurses can support anticipatory grief by encouraging open communication and helping the family address unfinished matters.

#258 · Psychosocial Integrity

What are common physical signs that death is likely within hours in a palliative care client?

Answer

Signs include mottling of the skin on the extremities, cool skin, decreased urine output, irregular or Cheyne-Stokes breathing patterns, and audible respiratory secretions sometimes called a death rattle. Level of consciousness usually decreases and the client may become nonresponsive. The nurse should prepare the family for these changes and continue comfort measures such as repositioning and mouth care.

#259 · Psychosocial Integrity

How should a nurse respond when a dying client's family asks why the client has stopped eating and drinking?

Answer

The nurse should explain that decreased appetite and thirst are a normal part of the body slowing down near the end of life, not a sign of suffering or a cause of it. Forcing food or fluids at this stage can actually increase discomfort from aspiration or fluid overload. Mouth care and small comfort measures are more appropriate than pushing intake.

#260 · Psychosocial Integrity

What is the difference between a Do Not Resuscitate order and palliative or comfort care?

Answer

A Do Not Resuscitate order specifically addresses whether cardiopulmonary resuscitation will be attempted if the heart or breathing stops, but it does not limit other forms of treatment. Palliative or comfort care is a broader approach focused on symptom relief and quality of life rather than cure, and it may or may not include a Do Not Resuscitate order. A nurse should clarify which specific orders and goals of care are in place for each client.

#261 · Psychosocial Integrity

What is complicated grief and how does it differ from normal grief?

Answer

Complicated grief is a prolonged and intense grief reaction that does not ease over time and significantly interferes with daily functioning, sometimes lasting well beyond a year. Normal grief typically softens gradually even though waves of sadness can recur. A nurse who notices a family member unable to resume basic functioning long after a loss should suggest referral to grief counselling or mental health support.

#262 · Psychosocial Integrity

What is the nurse's priority action when a client in crisis states they have a specific plan to end their life?

Answer

The nurse must ensure immediate safety by not leaving the client alone, removing access to means such as sharps or medications, and notifying the appropriate mental health or crisis team right away. A specific plan with intent and means represents high risk and requires urgent intervention rather than routine follow-up. The nurse should also document the disclosure and the actions taken in detail.

#263 · Psychosocial Integrity

What communication technique should a nurse use first when de-escalating an agitated client?

Answer

The nurse should speak in a calm, low tone, use simple clear language, and give the client physical space while maintaining a nonthreatening posture. Active listening and acknowledging the client's feelings often reduce agitation faster than trying to argue or reason immediately. Only after safety is established should the nurse attempt problem solving with the client.

#264 · Psychosocial Integrity

What is the difference between therapeutic use of silence and an awkward pause in a nurse-client conversation?

Answer

Therapeutic silence gives the client time to process emotions and organize thoughts, and the nurse remains attentive and present during it. An awkward pause happens when the nurse is uncomfortable or distracted and the client senses the conversation has stalled rather than being given space. Skilled use of silence is an active listening technique, not simply an absence of talking.

#265 · Psychosocial Integrity

Why should a nurse avoid saying 'everything will be fine' to a client facing a serious diagnosis?

Answer

This statement offers false reassurance because the nurse cannot guarantee the outcome, and it can shut down the client's willingness to express fear or ask difficult questions. A more therapeutic response acknowledges the client's feelings and invites them to share more, such as asking what worries them most right now. Honest, open communication builds trust better than empty reassurance.

#266 · Psychosocial Integrity

What is the key difference between using a professional interpreter and using a client's family member to translate during a health assessment?

Answer

A professional interpreter provides accurate, neutral translation and maintains confidentiality, while a family member may filter, summarize, or feel uncomfortable relaying sensitive information such as abuse or reproductive health questions. Family members are also not necessarily trained in medical terminology, which increases the risk of miscommunication. A trained interpreter should be used whenever available, especially for sensitive topics.

#267 · Psychosocial Integrity

What is cultural safety, and how does it differ from cultural awareness alone?

Answer

Cultural safety requires the nurse to actively reflect on their own biases and power position and to create care environments where clients from other cultures do not feel judged or diminished. Cultural awareness alone is only recognizing that cultural differences exist, without necessarily changing behaviour. Nurses working with Indigenous clients in Canada are expected to practice cultural safety because of documented health system barriers this population has faced.

#268 · Psychosocial Integrity

What is the difference between negative symptoms and positive symptoms in schizophrenia, and give one example of each?

Answer

Positive symptoms are experiences added to normal functioning, such as hallucinations or delusions. Negative symptoms are reductions in normal functioning, such as flat affect, social withdrawal, or reduced motivation. Both symptom types affect the client's daily functioning, but negative symptoms are often harder to treat with medication.

#269 · Psychosocial Integrity

What is a healthy coping mechanism the nurse can teach a client experiencing chronic stress, and why is it effective?

Answer

Regular physical activity is an effective coping strategy because it reduces circulating stress hormones and releases endorphins that improve mood. Other effective strategies include structured relaxation techniques such as deep breathing, progressive muscle relaxation, and maintaining consistent sleep routines. The nurse should help the client identify a strategy that fits realistically into their daily life rather than suggesting a generic list.

#270 · Psychosocial Integrity

Why is it important for a nurse to assess a client's support system as part of a psychosocial assessment?

Answer

A strong support system, such as family, friends, or community groups, can improve coping, treatment adherence, and recovery outcomes, while an absent or unhealthy support system increases the risk of relapse or poor mental health outcomes. The nurse should ask specifically who the client can call on in a crisis rather than assuming family involvement exists. Identifying gaps allows the care team to connect the client with community resources before discharge.

#271 · Basic Care and Comfort

What is the correct way to describe a Stage 1 pressure injury?

Answer

Intact skin with a localized area of non-blanchable erythema that does not turn white when pressed. The area may feel warmer, cooler, firmer, or softer than surrounding tissue and the person may report pain or itching before any visible skin break occurs.

#272 · Basic Care and Comfort

How does a Stage 2 pressure injury differ from a Stage 1 injury?

Answer

Stage 2 involves partial thickness loss of the dermis, presenting as a shallow open ulcer with a pink or red wound bed, or an intact or ruptured serum filled blister. Unlike Stage 1, the skin surface is actually broken rather than simply discoloured.

#273 · Basic Care and Comfort

What defines a Stage 3 pressure injury?

Answer

Full thickness tissue loss where subcutaneous fat may be visible, but bone, tendon, and muscle are not exposed. Slough may be present and undermining or tunnelling can occur, and depth varies by body location depending on how much adipose tissue is present there.

#274 · Basic Care and Comfort

What makes a pressure injury unstageable rather than Stage 3 or 4?

Answer

The wound bed is covered by slough or eschar so thick or adherent that the true depth of tissue loss cannot be confirmed. Once enough slough or eschar is removed to expose the wound bed, the injury can then be staged accurately as Stage 3 or Stage 4.

#275 · Basic Care and Comfort

How often should an immobile client's position be changed to prevent pressure injuries, and why is this timing chosen?

Answer

Repositioning is generally done at least every two hours for clients in bed, since capillary closing pressure is exceeded within roughly that timeframe and prolonged compression cuts off blood flow to tissue over bony prominences. Clients in a chair usually need weight shifted or repositioning every hour because seated pressure over the ischial tuberosities is more concentrated.

#276 · Basic Care and Comfort

Why should a practical nurse avoid massaging reddened areas over bony prominences?

Answer

Massaging tissue that is already showing non-blanchable redness can further damage capillaries that are already compromised and worsen underlying tissue injury. Instead the nurse should relieve pressure on the area, document the finding, and reposition the client.

#277 · Basic Care and Comfort

What is the purpose of keeping the head of the bed at 30 degrees or lower whenever the client's condition allows?

Answer

Elevating the head of the bed higher than 30 degrees increases shearing force as the body slides down against the sheets, which drags tissue layers in opposite directions and damages small blood vessels. Keeping the elevation low, along with using a trapeze or draw sheet to reposition, reduces this shear risk.

#278 · Basic Care and Comfort

A client has been on bedrest for two weeks. What early cardiovascular change should the nurse expect from prolonged immobility?

Answer

Orthostatic hypotension is common because immobility reduces vascular tone and blood pooling occurs more easily in the lower limbs when the client sits or stands. The nurse should raise the head of the bed gradually and dangle the client's legs before standing to prevent dizziness or falls.

#279 · Basic Care and Comfort

Why are immobile clients at higher risk for deep vein thrombosis?

Answer

Lack of muscle contraction in the legs slows venous return, allowing blood to pool and stasis to develop, which is one of the three factors in Virchow's triad along with vessel wall injury and hypercoagulability. Nurses reduce this risk with leg exercises, early ambulation, compression stockings, and prescribed anticoagulants.

#280 · Basic Care and Comfort

What is footdrop and how is it prevented in an immobile client?

Answer

Footdrop is a contracture of the ankle in which the foot remains plantar flexed and the client cannot dorsiflex it, making walking difficult once mobility returns. It is prevented by using a footboard, high top sneakers, or ankle splints and by performing range of motion exercises regularly.

#281 · Basic Care and Comfort

What is the difference between active and passive range of motion exercises?

Answer

Active range of motion is performed by the client using their own muscle strength without assistance, while passive range of motion is performed by the nurse moving the client's joints through their range because the client cannot do so independently. Both types help prevent contractures and maintain joint mobility.

#282 · Basic Care and Comfort

When teaching a client to use a cane, on which side should the cane be held?

Answer

The cane should be held in the hand on the stronger, unaffected side of the body. The client advances the cane and the weaker leg together, then steps through with the stronger leg, which keeps two points of support on the ground at all times.

#283 · Basic Care and Comfort

What is the correct technique for measuring a client for axillary crutches?

Answer

There should be a gap of about two to three finger widths, roughly 4 to 5 cm, between the top of the crutch pad and the client's axilla when standing upright with shoes on. Hand grips should be adjusted so the elbows are slightly flexed at about 15 to 30 degrees, and weight should always rest on the hands, never the axillae, to avoid nerve damage.

#284 · Basic Care and Comfort

How does a two point gait pattern with crutches work?

Answer

The client moves one crutch and the opposite leg forward together, then moves the other crutch and remaining leg forward together, mimicking a normal walking pattern. This gait requires the client to bear at least partial weight on both legs.

#285 • Basic Care and Comfort

What is a key safety principle when assisting a client with a walker?

Answer

The walker should be picked up and moved forward a short distance, all four legs set down, and then the client steps into it rather than reaching too far ahead. The nurse should also check that all four rubber tips are intact and the height is set so the client's elbows are slightly bent when hands rest on the grips.

#286 • Basic Care and Comfort

A client reports not having had a bowel movement in four days and the abdomen is distended. What should the nurse assess before intervening?

Answer

The nurse should auscultate bowel sounds, palpate for a firm mass in the lower abdomen, and ask about usual bowel pattern, diet, fluid intake, activity level, and any medications such as opioids that slow motility. This assessment helps determine whether the problem is simple constipation or a possible impaction requiring digital removal.

#287 · Basic Care and Comfort

What nursing measures help prevent constipation in a client with reduced mobility?

Answer

Increasing fluid intake, adding fibre rich foods, encouraging whatever activity or ambulation is safe for the client, and establishing a regular toileting schedule that respects the client's usual pattern all support normal bowel function. Privacy and an upright sitting position on the toilet or commode also make defecation easier.

#288 · Basic Care and Comfort

What is a Texas catheter or condom catheter used for and what is one key nursing precaution with it?

Answer

It is an external urinary collection device applied over the penis for a male client with urinary incontinence, used instead of an indwelling catheter to lower infection risk. The nurse must check that it is not applied too tightly, since excessive constriction can impair circulation and cause skin breakdown.

#289 · Basic Care and Comfort

A client's intake and output record shows 1200 mL oral fluids, 250 mL IV fluids, and a 200 mL ice chip serving that melts to half its volume. What is the total intake to record?

Answer

Ice chips are recorded at half their measured volume because they melt to about half the original amount, so 200 mL of ice chips counts as 100 mL of intake. Adding 1200 mL oral, 250 mL IV, and 100 mL from ice chips gives a total intake of 1550 mL.

#290 · Basic Care and Comfort

What is considered a minimally acceptable urine output for an adult, and why does this number matter?

Answer

Urine output should be at least 30 mL per hour, or roughly 0.5 mL per kilogram of body weight per hour, in an adult client. Output below this level over several consecutive hours can signal dehydration, kidney impairment, or inadequate perfusion and should be reported promptly.

#291 · Basic Care and Comfort

What is the purpose of using distraction, guided imagery, or slow rhythmic breathing with a client in pain?

Answer

These are nonpharmacological comfort measures that work by shifting the client's attention away from the pain stimulus or by promoting relaxation, which can lower perceived pain intensity and reduce muscle tension. They are used alongside, not instead of, prescribed analgesics for moderate to severe pain.

#292 · Basic Care and Comfort

When is applying heat contraindicated for a client with a musculoskeletal injury?

Answer

Heat should not be applied in the first 24 to 48 hours after an acute injury because it increases blood flow and can worsen swelling and bleeding into the tissue. Cold therapy is used instead during this early period to constrict vessels and limit edema, with heat reserved for later to promote healing and relax muscles.

#293 · Basic Care and Comfort

In palliative care, what is the priority nursing focus when a dying client develops noisy, wet sounding respirations sometimes called a death rattle?

Answer

The priority is comfort for the client rather than aggressive suctioning, since the sound is caused by pooled secretions the client can no longer clear and suctioning rarely helps and can cause distress. Repositioning the client onto their side and reducing fluid intake as ordered can help, and the nurse should reassure family that the client is generally not experiencing distress from this sound.

#294 · Basic Care and Comfort

What are the four standard thickened liquid levels used in dysphagia diets under IDDSI?

Answer

The International Dysphagia Diet Standardisation Initiative levels are thin, slightly thick, mildly thick, moderately thick, and extremely thick, numbered 0 through 4. A speech language pathologist orders the specific level based on swallow assessment findings. The practical nurse must serve fluids at the exact prescribed thickness to prevent aspiration.

#295 · Basic Care and Comfort

A client on a dysphagia diet tucks their chin when swallowing. What is this technique called and why is it used?

Answer

This is called the chin tuck maneuver. It narrows the airway entrance and widens the space at the back of the throat, which slows the bolus and reduces the risk of food or liquid entering the airway. It is commonly taught to clients with an impaired swallow reflex.

#296 · Basic Care and Comfort

How should a nurse position a client before, during, and after tube feeding to reduce aspiration risk?

Answer

The head of the bed should be elevated to at least 30 to 45 degrees before starting the feed. This position should be maintained during the feeding and for at least 30 to 60 minutes afterward. Keeping the client upright allows gravity to help move formula through the stomach and lowers the chance of reflux into the lungs.

#297 · Basic Care and Comfort

Before giving an intermittent tube feeding, what should the nurse check first and why?

Answer

The nurse should verify correct tube placement and check gastric residual volume according to agency policy. Confirming placement prevents formula from being instilled into the lungs if the tube has migrated. A high residual volume may signal delayed gastric emptying and the need to hold or slow the feeding.

#298 · Basic Care and Comfort

A client drank 240 mL of juice, ate 150 mL of soup, and received 500 mL of intravenous fluid. What is the total intake in millilitres?

Answer

Add all liquid sources together: 240 mL plus 150 mL plus 500 mL equals 890 mL. Intravenous fluids, oral fluids, and liquid foods such as soup or gelatin all count toward total fluid intake. Solid food that is not liquid at room temperature is not included.

#299 · Basic Care and Comfort

How many millilitres are in one ounce for the purpose of intake and output calculations?

Answer

One ounce equals approximately 30 millilitres. This conversion is used whenever a client reports fluid intake in ounces or cups so it can be recorded in the metric units required on the intake and output record. For example, an 8 ounce cup of water equals about 240 mL.

#300 · Basic Care and Comfort

What output sources must be included on a complete 24 hour intake and output record?

Answer

Output includes urine, liquid stool, vomitus, wound drainage, nasogastric or tube drainage, and any fluid removed by suction. Each source should be measured separately and totalled at the end of the shift or day. Accurate output measurement helps detect fluid imbalance and guides fluid replacement decisions.

#301 · Basic Care and Comfort

What urine output level defines oliguria in an adult client?

Answer

Oliguria is defined as urine output of less than 30 mL per hour or less than approximately 400 to 500 mL over 24 hours in an adult. It can indicate dehydration, kidney impairment, or an obstruction in the urinary tract. This finding should be reported promptly for further assessment.

#302 · Basic Care and Comfort

What are three early signs of dehydration the nurse should assess for in an older adult client?

Answer

Early signs include dry mucous membranes, decreased skin turgor, and concentrated dark yellow urine. Older adults may also show confusion or increased fatigue before other signs appear because thirst sensation declines with age. Vital sign changes such as increased heart rate and low blood pressure can develop as dehydration progresses.

#303 · Basic Care and Comfort

What is the correct technique for measuring residual urine volume after a client voids using a bladder scanner?

Answer

The nurse has the client void completely first, then places the ultrasound bladder scanner probe midline on the lower abdomen about 4 cm above the pubic bone. The scanner calculates the volume of urine remaining in the bladder. A postvoid residual above 100 to 150 mL may indicate incomplete bladder emptying and should be reported.

#304 · Basic Care and Comfort

How often should an indwelling urinary catheter drainage bag be kept positioned to prevent infection?

Answer

The drainage bag must always be kept below the level of the bladder and off the floor. This position uses gravity to promote drainage and prevents urine from flowing backward into the bladder, which increases infection risk. The tubing should also be kept free of kinks and dependent loops.

#305 · Basic Care and Comfort

What routine catheter care should the nurse provide for a client with an indwelling urinary catheter?

Answer

The nurse cleans the perineal area and the first several centimetres of catheter closest to the meatus with soap and water during daily hygiene and after bowel movements. Vigorous scrubbing or use of antiseptic solutions on the catheter is not recommended. Hand hygiene before and after care and maintaining a closed drainage system are essential to prevent catheter associated urinary tract infection.

#306 · Basic Care and Comfort

What is intermittent self catheterization and which clients commonly use it?

Answer

Intermittent self catheterization involves inserting a straight catheter to empty the bladder on a scheduled basis, then removing it, rather than leaving a catheter in place continuously. It is commonly used by clients with neurogenic bladder from spinal cord injury or multiple sclerosis. This method lowers the risk of catheter associated infection compared with an indwelling catheter.

#307 · Basic Care and Comfort

What nonpharmacological bladder training strategies can help a client with urge incontinence?

Answer

Bladder training includes scheduled voiding at set intervals, gradually lengthening the time between voids, and pelvic floor muscle exercises such as Kegel exercises. Limiting bladder irritants like caffeine and alcohol also helps reduce urgency. These strategies aim to increase bladder capacity and improve voluntary control over time.

#308 · Basic Care and Comfort

What position best facilitates defecation and should be encouraged for a client using a bedpan or toilet?

Answer

A high Fowler position or sitting upright with the hips flexed and knees slightly higher than the hips mimics a natural squatting posture. This position uses gravity and reduces the effort needed to pass stool. When a client must use a bedpan, raising the head of the bed as much as tolerated helps achieve this position.

#309 • Basic Care and Comfort

What dietary and fluid measures help prevent constipation in a client with limited mobility?

Answer

Increasing dietary fibre from sources such as whole grains, fruits, and vegetables adds bulk to stool. Adequate fluid intake, generally around 1500 to 2000 mL per day unless restricted, keeps stool soft and easier to pass. Encouraging whatever activity or mobility the client can tolerate also stimulates peristalsis.

#310 • Basic Care and Comfort

Before administering a cleansing enema, what client assessment finding would make the nurse withhold the enema and notify the care provider?

Answer

The nurse should withhold the enema and notify the care provider if the client has severe abdominal pain, rectal bleeding, or a recent bowel or rectal surgery. These findings could indicate a condition where an enema would worsen tissue damage or mask a serious complication. Suspected bowel obstruction is another reason to hold the enema and seek further assessment.

#311 · Basic Care and Comfort

What are the classic signs and symptoms of fecal impaction the nurse should recognize?

Answer

Signs include absence of a normal bowel movement for several days, abdominal distension and discomfort, and leakage of liquid stool around the impacted mass, sometimes mistaken for diarrhea. The client may also report a feeling of rectal fullness or pressure. Digital rectal examination can confirm the presence of hardened stool in the rectum.

#312 · Basic Care and Comfort

What is the purpose of using an alternating pressure mattress or repositioning schedule for an immobile client?

Answer

These interventions redistribute pressure over bony prominences to maintain blood flow to the skin and underlying tissue. Prolonged unrelieved pressure can lead to tissue ischemia and pressure injury formation within a short period. A repositioning schedule of at least every two hours is a standard baseline, adjusted based on individual skin assessment and risk level.

#313 · Basic Care and Comfort

What complications of prolonged immobility should the nurse monitor for besides pressure injuries?

Answer

Prolonged immobility increases the risk of deep vein thrombosis, muscle atrophy and joint contractures, orthostatic hypotension, constipation, and respiratory complications such as atelectasis or pneumonia from pooled secretions. Range of motion exercises, early mobilization, and deep breathing exercises help reduce these risks. Frequent position changes also support circulation and lung expansion.

#314 · Basic Care and Comfort

What nonpharmacological comfort measures can the nurse use to help a client fall asleep in an unfamiliar hospital environment?

Answer

Useful measures include dimming lights, reducing noise, offering a warm blanket or backrub, and maintaining the client's usual bedtime routine as closely as possible. Limiting caffeine intake in the evening and clustering nursing care to allow uninterrupted rest periods also support sleep. A comfortable room temperature and positioning aligned with the client's preference further promote rest.

#315 · Basic Care and Comfort

In palliative care, what nonpharmacological approach helps manage dyspnea in a dying client?

Answer

Positioning the client upright or in a high Fowler position and directing a small fan or open window airflow toward the face can ease the subjective sensation of breathlessness. Reducing environmental stimulation and staying calmly present with the client also decreases anxiety that can worsen dyspnea. These measures are used alongside any prescribed medications rather than replacing them.

#316 · Pharmacological and Parenteral Therapies

What is the basic formula for calculating an IV flow rate in drops per minute?

Answer

Drops per minute equals the volume in mL divided by the time in minutes, multiplied by the drop factor of the tubing in gtt/mL. The result is always rounded to the nearest whole drop since a fraction of a drop cannot be counted.

#317 · Pharmacological and Parenteral Therapies

How do you calculate an IV pump rate in mL per hour when given a total volume and infusion time?

Answer

Divide the total volume to be infused in mL by the total time in hours. For example, 1000 mL ordered over 8 hours equals 125 mL per hour, which is the rate entered directly into the infusion pump.

#318 · Pharmacological and Parenteral Therapies

What are the traditional rights of medication administration every practical nurse must check?

Answer

The traditional rights are right patient, right medication, right dose, right route, right time, and right documentation. Many Canadian programs now add right reason and right to refuse to reflect current safe practice standards.

#319 · Pharmacological and Parenteral Therapies

Which two patient identifiers must a nurse use before administering any medication?

Answer

The nurse must use at least two identifiers such as the patient's full name and date of birth, or name and hospital identification number. The room number or bed number is never an acceptable identifier because patients can be moved.

#320 · Pharmacological and Parenteral Therapies

What is the formula for calculating a dosage from a medication order using desired dose, dose on hand, and quantity on hand?

Answer

The formula is desired dose divided by dose on hand, multiplied by the quantity on hand, often remembered as D over H times Q. This formula works for tablets, capsules, and liquid oral medications alike.

#321 · Pharmacological and Parenteral Therapies

A patient develops hives, facial swelling, and wheezing minutes after a new antibiotic infusion starts. What should the nurse do first?

Answer

The nurse should stop the infusion immediately and keep the IV line open with normal saline while calling for help. These are signs of an anaphylactic reaction, and rapid response with epinephrine and airway support may be needed.

#322 · Pharmacological and Parenteral Therapies

What is the difference between a peripheral IV line and a central venous line?

Answer

A peripheral IV line is inserted into a smaller vein in the arm or hand and is used for short term therapy and lower osmolarity fluids. A central venous line terminates in a large central vein near the heart and can safely deliver irritating or highly concentrated solutions such as parenteral nutrition.

#323 · Pharmacological and Parenteral Therapies

What should the nurse check before hanging a unit of packed red blood cells?

Answer

The nurse must verify the patient identity, blood type and Rh compatibility, unit number, and expiry date with a second qualified person according to facility policy. Vital signs are also taken before starting the transfusion to establish a baseline.

#324 · Pharmacological and Parenteral Therapies

What is the priority nursing action during the first fifteen minutes of a blood transfusion?

Answer

The nurse stays with the patient and monitors closely for signs of a transfusion reaction such as fever, chills, back pain, or hives. Most severe reactions occur within this early period, so the infusion rate is kept slow at the start.

#325 · Pharmacological and Parenteral Therapies

What is the first action if a patient shows signs of a hemolytic blood transfusion reaction?

Answer

The nurse stops the transfusion immediately and keeps the IV open with normal saline through new tubing. The reaction and the remaining blood unit are then reported to the blood bank and physician for further testing.

#326 · Pharmacological and Parenteral Therapies

Why is a dedicated line required for total parenteral nutrition when possible?

Answer

A dedicated line reduces the risk of contamination and incompatibility since TPN solutions are high in dextrose and prone to bacterial growth. Sharing the line with other medications also increases the risk of clotting or reaction with the concentrated solution.

#327 · Pharmacological and Parenteral Therapies

What blood glucose complication should the nurse monitor for when a patient receiving TPN has the infusion stopped abruptly?

Answer

Abrupt discontinuation of TPN can cause rebound hypoglycemia because the body has adjusted to a high steady glucose load. TPN should be tapered off gradually, and blood glucose should be monitored closely during the change.

#328 · Pharmacological and Parenteral Therapies

What does it mean when an IV site is described as infiltrated?

Answer

Infiltration means IV fluid has leaked into the surrounding tissue rather than staying in the vein, causing swelling, coolness, and discomfort at the site. The infusion should be stopped and the IV restarted in a new location.

#329 · Pharmacological and Parenteral Therapies

How does extravasation differ from simple infiltration?

Answer

Extravasation occurs when the leaking fluid is a vesicant medication capable of causing tissue damage or necrosis, not just a plain solution. It requires immediate action to limit tissue injury, which may include stopping the infusion, aspirating residual drug, and notifying the prescriber for a specific antidote protocol.

#330 · Pharmacological and Parenteral Therapies

What are the classic signs of phlebitis at an IV insertion site?

Answer

Phlebitis presents with redness, warmth, tenderness, and a palpable cord along the vein at or above the insertion site. The IV should be discontinued and a warm compress applied, with the site reassessed regularly afterward.

#331 · Pharmacological and Parenteral Therapies

What is the standard formula for calculating a safe pediatric or weight based medication dose?

Answer

The ordered dose per kilogram is multiplied by the patient's actual weight in kilograms to find the total safe dose. The result is then compared against the prescribed order to confirm the dose falls within a safe range before administration.

#332 · Pharmacological and Parenteral Therapies

How do you convert a patient's weight from pounds to kilograms for dosage calculation?

Answer

Divide the weight in pounds by 2.2 to get the weight in kilograms. This conversion is essential before applying any weight based dosing formula since most drug references list safe doses in milligrams per kilogram.

#333 · Pharmacological and Parenteral Therapies

What is the correct action when a nurse discovers a medication error has occurred?

Answer

The nurse assesses the patient for harm first, notifies the prescriber and charge nurse, then documents the incident factually in the health record and completes an incident report per facility policy. The error is never hidden, since patient safety and honest disclosure come first.

#334 · Pharmacological and Parenteral Therapies

Why should opioid pain medications be tapered rather than stopped abruptly after long term use?

Answer

Abrupt discontinuation after prolonged use can trigger withdrawal symptoms such as agitation, sweating, tachycardia, and abdominal cramping. Tapering the dose gradually allows the body to adjust and reduces the severity of withdrawal effects.

#335 · Pharmacological and Parenteral Therapies

What is the priority assessment before administering a scheduled dose of an opioid analgesic?

Answer

The nurse assesses the patient's respiratory rate and level of sedation before giving the dose, since opioids can cause respiratory depression. A respiratory rate below 10 to 12 breaths per minute or significant sedation is a reason to hold the dose and notify the prescriber.

#336 · Pharmacological and Parenteral Therapies

What is the antidote used to reverse opioid induced respiratory depression?

Answer

Naloxone is the antidote given to reverse opioid overdose and respiratory depression. Because naloxone has a shorter duration of action than many opioids, the patient must be monitored closely for return of sedation after the initial dose wears off.

#337 · Pharmacological and Parenteral Therapies

What does the term therapeutic index describe about a medication?

Answer

The therapeutic index describes the margin between a medication's effective dose and its toxic dose. Drugs with a narrow therapeutic index, such as warfarin or digoxin, require close monitoring of blood levels because toxicity can occur close to the therapeutic dose.

#338 · Pharmacological and Parenteral Therapies

What is the purpose of checking a trough level before giving an IV antibiotic such as vancomycin?

Answer

A trough level is drawn just before the next scheduled dose to measure the lowest drug concentration in the blood. This helps confirm the dose is high enough to remain effective while avoiding accumulation that could cause toxicity such as kidney damage.

#339 · Pharmacological and Parenteral Therapies

What action should the nurse take if two medications are found to be incompatible in the same IV line?

Answer

The nurse should flush the line with a compatible solution such as normal saline between the two medications, or use a separate lumen if the line is multi lumen. Administering incompatible drugs together can cause precipitation, reduced drug effectiveness, or line occlusion.

#340 · Pharmacological and Parenteral Therapies

What is the correct method for calculating the concentration of a solution in milligrams per milliliter?

Answer

Divide the total amount of drug in milligrams by the total volume of solution in milliliters to find the concentration. This value is then used to calculate the exact volume needed to deliver a prescribed dose.

#341 · Pharmacological and Parenteral Therapies

Why is it important to use a filter needle when withdrawing medication from a glass ampoule?

Answer

A filter needle prevents small glass particles from being drawn into the syringe when the ampoule neck is broken open. The filter needle is changed for a plain needle before injecting the medication into the patient.

#342 · Pharmacological and Parenteral Therapies

What is the sixth right approach the practical nurse uses beyond the original five rights of medication administration?

Answer

Many Canadian nursing programs teach a sixth right, right documentation, in addition to right patient, medication, dose, route, and time. Documenting immediately after administration, not before, confirms the medication was actually given and prevents duplicate dosing.

#343 · Pharmacological and Parenteral Therapies

What nursing action reduces the risk of a central line associated bloodstream infection during dressing changes?

Answer

The nurse uses strict aseptic technique, cleanses the site with chlorhexidine using a back and forth or circular motion as per policy, and lets the skin air dry before applying a new sterile dressing. Hand hygiene before and after the procedure is essential since central lines bypass the skin's natural barrier.

#344 · Pharmacological and Parenteral Therapies

What are the signs of hypoglycemia the practical nurse should teach a client on insulin to watch for?

Answer

Sweating, shakiness, confusion, tachycardia, and hunger are common signs of hypoglycemia. The nurse should teach the client to check blood glucose immediately if these occur and to treat with 15 grams of fast-acting carbohydrate. Blood glucose should be rechecked in 15 minutes and treatment repeated if still low.

#345 · Pharmacological and Parenteral Therapies

Why is insulin classified as a high-alert medication?

Answer

Insulin has a narrow margin between a therapeutic dose and a dose that causes serious harm. A small dosing error or mix up between insulin types can cause severe hypoglycemia, seizures, or death. Independent double checks are required before administration in most facilities.

#346 · Pharmacological and Parenteral Therapies

What lab value is used to monitor a client on warfarin therapy?

Answer

The international normalized ratio, known as INR, is used to monitor warfarin therapy. The therapeutic range for most clients is generally 2 to 3, though it may be higher for clients with mechanical heart valves. A rising INR above therapeutic range increases bleeding risk.

#347 · Pharmacological and Parenteral Therapies

What is the antidote for warfarin toxicity?

Answer

Vitamin K, also called phytonadione, is the antidote for warfarin toxicity. In cases of severe bleeding, fresh frozen plasma or prothrombin complex concentrate may also be given to reverse the effect more quickly. The nurse should monitor the client for signs of bleeding such as bruising, blood in urine, or dark stools.

#348 · Pharmacological and Parenteral Therapies

What lab value is monitored for a client receiving intravenous heparin?

Answer

Activated partial thromboplastin time, known as aPTT, is monitored for clients on intravenous heparin. The therapeutic range is usually 1.5 to 2.5 times the normal control value. The nurse should have protamine sulfate available as the reversal agent in case of overdose or bleeding.

#349 · Pharmacological and Parenteral Therapies

What is the first action if a client develops signs of a transfusion reaction?

Answer

The nurse should stop the transfusion immediately and keep the intravenous line open with normal saline using new tubing. The vital signs are then assessed and the primary care provider and blood bank are notified right away. The remaining blood product and tubing are saved and returned to the lab for testing.

#350 · Pharmacological and Parenteral Therapies

Which intravenous solution must always be used to prime tubing before and after a blood transfusion?

Answer

Normal saline, which is 0.9 percent sodium chloride, is the only solution used to prime tubing for blood products. Dextrose solutions cause red blood cells to hemolyze and lactated Ringer's contains calcium that can cause clots to form in the blood product. The nurse should never add medications directly into a blood transfusion line.

#351 · Pharmacological and Parenteral Therapies

How soon after starting a blood transfusion should the nurse remain at the bedside, and why?

Answer

The nurse should stay with the client for the first 15 minutes of the transfusion. Most severe transfusion reactions, including hemolytic and anaphylactic reactions, occur within this early period. Vital signs are taken before the transfusion, at 15 minutes, and then per facility policy throughout the infusion.

#352 · Pharmacological and Parenteral Therapies

What is the maximum time a unit of packed red blood cells can hang once removed from refrigeration?

Answer

A unit of packed red blood cells must be infused within 4 hours of leaving the blood bank refrigerator. Bacterial growth risk increases significantly once blood warms above refrigerator temperature. If the infusion cannot be completed within this window, the remaining product must be discarded and not returned to the fridge.

#353 · Pharmacological and Parenteral Therapies

What are early signs of infiltration at a peripheral intravenous site?

Answer

Infiltration presents with swelling, coolness, and pallor at the insertion site, along with a slowed infusion rate. There is no redness or warmth, which distinguishes it from phlebitis. The nurse should stop the infusion, remove the catheter, elevate the limb, and restart the line in a different site.

#354 · Pharmacological and Parenteral Therapies

How does phlebitis at an intravenous site differ from infiltration?

Answer

Phlebitis presents with redness, warmth, and tenderness along the vein, sometimes with a palpable cord. Infiltration instead causes coolness, swelling, and pallor without warmth or redness. Both require the intravenous catheter to be removed and a new site established elsewhere.

#355 · Pharmacological and Parenteral Therapies

A client's IV is infusing at 125 mL per hour using tubing with a drop factor of 15 gtt per mL. What is the flow rate in drops per minute?

Answer

Using the formula volume in mL divided by time in minutes multiplied by the drop factor, 125 mL divided by 60 minutes equals about 2.08 mL per minute. Multiplying by 15 gtt per mL gives approximately 31 drops per minute. The nurse would round this to 31 gtt per minute when regulating a gravity infusion.

#356 · Pharmacological and Parenteral Therapies

What are signs of fluid volume overload the nurse should watch for during rapid intravenous infusion?

Answer

Signs include shortness of breath, crackles in the lungs, jugular vein distension, and rising blood pressure. Peripheral edema and a rapid weight gain may also be present. The nurse should slow the infusion, raise the head of the bed, and notify the primary care provider immediately.

#357 · Pharmacological and Parenteral Therapies

Why must potassium chloride never be given as an intravenous push?

Answer

Rapid intravenous administration of potassium chloride can cause fatal cardiac dysrhythmias including cardiac arrest. It must always be diluted and administered as a slow infusion using an infusion pump, with the rate not exceeding facility-approved limits. Continuous cardiac monitoring is recommended for concentrated potassium infusions.

#358 · Pharmacological and Parenteral Therapies

What is a key nursing responsibility before accessing a central venous catheter such as a peripherally inserted central catheter (PICC)?

Answer

The nurse must confirm correct catheter tip placement was verified by x-ray before initial use and check patency by aspirating for blood return. Strict aseptic technique is used when accessing the line to prevent catheter-related bloodstream infection. The line is then flushed with normal saline before and after use according to facility protocol.

#359 · Pharmacological and Parenteral Therapies

What complication should the nurse suspect if a client with a central line suddenly develops chest pain, shortness of breath, and a drop in oxygen saturation?

Answer

An air embolism should be suspected in this situation. The nurse should immediately clamp the catheter, place the client in the left lateral Trendelenburg position, administer oxygen, and call for emergency help. This position traps air in the right atrium away from the pulmonary valve.

#360 · Pharmacological and Parenteral Therapies

What action should the nurse take if a central venous catheter site shows redness, warmth, and purulent drainage?

Answer

This suggests a catheter-related infection, and the nurse should notify the primary care provider promptly. Blood cultures are typically drawn, and the catheter may need to be removed and cultured depending on severity. The insertion site should continue to be monitored and documented using facility assessment tools.

#361 · Pharmacological and Parenteral Therapies

Why is total parenteral nutrition (TPN) always administered through a central line rather than a peripheral vein?

Answer

TPN solutions have a very high osmolarity due to their high concentrations of dextrose and amino acids. Infusing this hypertonic solution into a peripheral vein would cause vein damage, phlebitis, and sclerosis. Central veins have higher blood flow that rapidly dilutes the solution and prevents this vessel damage.

#362 · Pharmacological and Parenteral Therapies

What should the nurse do if a bag of total parenteral nutrition runs out before the next bag is available?

Answer

The nurse should hang a bag of 10 percent dextrose in water at the same rate as the TPN was infusing. This prevents rebound hypoglycemia, which can occur because the client's body has adjusted to a high steady glucose load. The primary care provider should be notified so a new TPN bag can be obtained promptly.

#363 · Pharmacological and Parenteral Therapies

What metabolic complication should the nurse monitor for when a client first starts total parenteral nutrition?

Answer

Hyperglycemia is a common early complication because TPN solutions contain high concentrations of dextrose. Blood glucose is typically checked every 4 to 6 hours during the initiation phase. The nurse should also watch for refeeding syndrome, which involves dangerous drops in phosphate, potassium, and magnesium.

#364 · Pharmacological and Parenteral Therapies

What is the nursing priority before administering an opioid analgesic to a client?

Answer

The nurse must assess the client's respiratory rate and level of sedation before giving an opioid, since respiratory depression is the most serious adverse effect. An opioid should generally be withheld and the prescriber notified if the respiratory rate is below 12 breaths per minute. Pain level and prior response to the medication should also be reviewed.

#365 · Pharmacological and Parenteral Therapies

What is the antidote for opioid overdose and how is it given?

Answer

Naloxone is the antidote for opioid overdose and reverses respiratory depression within minutes. It can be given intravenously, intramuscularly, or intranasally depending on the setting and available formulation. Because naloxone's effect can wear off before the opioid does, the client must be monitored closely for return of respiratory depression.

#366 · Pharmacological and Parenteral Therapies

What should the nurse teach a client using a patient-controlled analgesia (PCA) pump about who can press the button?

Answer

Only the client should press the PCA button to self-administer pain medication, never family members or staff. The PCA pump is designed with built-in safety limits so that if the client becomes too sedated, they will not be able to press the button again. This design prevents accidental overdose from someone else dosing on the client's behalf.

#367 · Pharmacological and Parenteral Therapies

What are the classic signs of an allergic drug reaction the nurse should assess for after giving a new medication?

Answer

Signs include urticaria, itching, flushing, and swelling of the face, lips, or tongue. More severe reactions can progress to wheezing, stridor, and a sudden drop in blood pressure, indicating anaphylaxis. The medication should be stopped immediately if signs appear and the primary care provider notified without delay.

#368 · Pharmacological and Parenteral Therapies

What are the six rights the nurse should verify before administering any medication?

Answer

The six rights are right client, right medication, right dose, right route, right time, and right documentation. Some frameworks add right reason and right response as additional checks. Verifying these rights at multiple points reduces the risk of medication administration errors.

#369 · Pharmacological and Parenteral Therapies

A prescription reads to give a medication at 0.25 mg per kg for a client weighing 68 kg. How many milligrams should the nurse administer?

Answer

Multiplying 0.25 mg by 68 kg gives a total dose of 17 mg. The nurse should double check this calculation against the maximum safe dose listed for the medication before administration.

Weight-based dosing errors are a common source of harm, so an independent check by a second nurse is recommended for high-alert drugs.

#370 · Pharmacological and Parenteral Therapies

Why should the nurse use tall man lettering awareness when reading medication labels such as hydrOXYzine and hydrALAZINE?

Answer

Tall man lettering highlights the different parts of look-alike drug names to prevent mix-ups between medications that sound or look similar.

HydrOXYzine is an antihistamine used for anxiety and itching, while hydrALAZINE is an antihypertensive. Confusing these two drugs could cause a dangerous drop in blood pressure or a missed treatment need.

#371 · Pharmacological and Parenteral Therapies

What nursing action reduces the risk of medication errors with look-alike, sound-alike drug names in a safe medication practice framework?

Answer

Independent double checks with a second qualified nurse are used for high-alert medications and look-alike, sound-alike drugs before administration. Reading the label at least three times, avoiding interruptions during preparation, and verifying with the client's medication administration record also reduce error risk. Facilities maintain lists of these confusable drug pairs to increase staff awareness.

#372 · Pharmacological and Parenteral Therapies

What is the correct order of the rights of medication administration a practical nurse must check before giving any drug?

Answer

The nurse verifies the right client, right drug, right dose, right route, right time, right documentation, and right reason. Many agencies now also add right to refuse and right assessment. Checking these rights at least three times, when pulling the drug, preparing it, and at the bedside, reduces administration errors.

#373 · Pharmacological and Parenteral Therapies

How do you calculate a pediatric weight-based dose when the order reads 10 mg/kg/day divided every 8 hours for a child weighing 22 kg?

Answer

Multiply the weight by the ordered dose per kilogram, so 22 kg times 10 mg equals 220 mg per day. Divide the daily total by the number of doses, so 220 mg divided by 3 doses equals about 73.3 mg per dose. Always compare this calculated dose against the safe range listed in a pediatric drug reference before administering.

#374 · Pharmacological and Parenteral Therapies

Why is body surface area sometimes used instead of body weight to calculate pediatric drug doses?

Answer

Body surface area correlates more closely with metabolic rate, cardiac output, and fluid distribution than weight alone, especially for chemotherapy and some antibiotics. It is calculated from height and weight using a nomogram or formula and expressed in square metres. This method is considered more precise for drugs with a narrow safety margin.

#375 · Pharmacological and Parenteral Therapies

What is a key safety reason for always double checking pediatric medication calculations with a second regulated provider?

Answer

Children have small body mass and immature organ systems, so a decimal point or unit error can quickly cause a toxic dose. An independent double check catches math mistakes before the drug reaches the child. Most Canadian hospital policies require this second check for high alert drugs such as insulin, opioids, and anticoagulants regardless of patient age.

#376 · Pharmacological and Parenteral Therapies

How do you calculate an IV infusion rate in millilitres per hour when a physician orders 1000 mL of normal saline over 8 hours?

Answer

Divide the total volume by the total time in hours, so 1000 mL divided by 8 hours equals 125 mL per hour. This rate is then programmed into an infusion pump or used to calculate the drop rate for gravity infusion. Always confirm the ordered rate matches what is safe for the client's cardiac and renal status.

#377 · Pharmacological and Parenteral Therapies

How do you calculate the drops per minute for a gravity IV infusion using a macrodrip set with a drop factor of 15 gtt/mL, infusing 100 mL over 60 minutes?

Answer

Multiply the volume by the drop factor and divide by the time in minutes, so 100 mL times 15 gtt divided by 60 minutes equals 25 drops per minute. The nurse counts drops in the drip chamber for one full minute to confirm the rate. This calculation is only needed when a pump is unavailable and gravity flow is used.

#378 · Pharmacological and Parenteral Therapies

What are the classic early signs of infiltration at a peripheral IV site?

Answer

Infiltration presents as swelling, coolness, and pallor around the insertion site along with discomfort and a sluggish or stopped infusion. Unlike phlebitis, there is no warmth or redness because the fluid is leaking into surrounding tissue rather than causing inflammation. The infusion should be stopped immediately, the catheter removed, and a new site started elsewhere.

#379 · Pharmacological and Parenteral Therapies

What distinguishes phlebitis from infiltration at a peripheral IV site?

Answer

Phlebitis is inflammation of the vein wall and presents with warmth, redness, tenderness, and a palpable cord along the vein path. Infiltration involves fluid leaking into surrounding tissue and presents as cool, pale, swollen skin without redness. Both require stopping the infusion and removing the catheter, but phlebitis is graded on a numeric scale and documented as such.

#380 · Pharmacological and Parenteral Therapies

What is extravasation and why is it more dangerous than simple infiltration?

Answer

Extravasation occurs when a vesicant medication, such as certain chemotherapy drugs or vasopressors, leaks into the surrounding tissue instead of the vein. Because the drug is caustic, it can cause blistering, tissue necrosis, and permanent damage rather than just swelling. The infusion must be stopped immediately, the specific antidote protocol followed if one exists, and the prescriber notified right away.

#381 · Pharmacological and Parenteral Therapies

What nursing actions are required before accessing a central venous access device for medication administration?

Answer

The nurse confirms correct catheter tip placement was verified by x-ray after insertion, checks for a blood return, and flushes with normal saline before and after each medication using the push-pause technique. The line is then flushed again after use and locked according to agency policy, often with heparin or saline depending on the device. Strict aseptic technique is used throughout because central lines bypass the body's normal skin barrier.

#382 · Pharmacological and Parenteral Therapies

What is the recommended technique for flushing a central venous access device to prevent catheter occlusion?

Answer

The nurse uses a push-pause technique, alternately pushing small amounts of saline and pausing, which creates turbulence that clears debris from the catheter lumen more effectively than a steady continuous push. A positive pressure technique is used when disconnecting the syringe to prevent blood backflow into the tip. Only prefilled or appropriately sized syringes rated for the catheter are used to avoid excessive pressure that could rupture the line.

#383 · Pharmacological and Parenteral Therapies

What are the signs and symptoms of a central line associated bloodstream infection?

Answer

The client may develop fever, chills, and hypotension without another identifiable source of infection, along with redness, warmth, or drainage at the exit site. Blood cultures drawn from the line and peripherally help confirm the diagnosis. Prevention relies on strict aseptic technique during insertion and dressing changes and daily assessment of whether the line is still needed.

#384 · Pharmacological and Parenteral Therapies

How often should a transparent dressing on a central venous access device typically be changed, and what triggers an earlier change?

Answer

Transparent semipermeable dressings are usually changed every 5 to 7 days according to agency policy. The dressing should be changed sooner if it becomes loose, soiled, damp, or if drainage is visible underneath. Gauze dressings used when there is drainage or bleeding are changed more frequently, typically every 48 hours.

#385 · Pharmacological and Parenteral Therapies

What action should the nurse take first if air embolism is suspected during central line care?

Answer

The nurse clamps the catheter immediately to stop further air entry and positions the client on their left side with the head lowered in Trendelenburg position. This positioning traps air in the right atrium away from the pulmonary outflow tract. Oxygen is applied and the prescriber is notified immediately as this is a medical emergency.

#386 · Pharmacological and Parenteral Therapies

What is the correct process for verifying a blood product before transfusion in Canada?

Answer

Two regulated healthcare providers independently verify the client's identity, blood type and crossmatch results, unit number, expiry date, and product type against the requisition and the client's identification band at the bedside. Vital signs are taken immediately before starting, at 15 minutes, and periodically throughout the transfusion. Any mismatch, no matter how minor, must be resolved before the unit is started.

#387 · Pharmacological and Parenteral Therapies

What is the nurse's priority action if a client develops fever, chills, and low back pain within the first 15 minutes of a blood transfusion?

Answer

The nurse stops the transfusion immediately, keeps the IV line open with normal saline through new tubing, and stays with the client to monitor vital signs. This presentation suggests an acute hemolytic transfusion reaction, which can be life threatening. The prescriber and blood bank are notified right away and the remaining blood unit and tubing are returned to the lab for testing.

#388 · Pharmacological and Parenteral Therapies

What is the maximum time a unit of packed red blood cells can safely hang once it leaves the blood bank refrigerator?

Answer

A unit of packed red blood cells must be completely infused within 4 hours of leaving controlled refrigeration because bacterial growth risk increases significantly after that window. If the infusion cannot be completed in time, it must be stopped and discarded rather than continued. This is why the nurse coordinates timing carefully before retrieving the unit from the blood bank.

#389 · Pharmacological and Parenteral Therapies

Why should normal saline be the only IV solution infused with blood products?

Answer

Dextrose solutions can cause red blood cells to hemolyze, and solutions containing calcium, such as Ringer's lactate, can cause clotting within the blood administration set. Normal saline is isotonic and compatible with blood components, so it is used to prime the tubing and as the only solution that may run alongside the transfusion. Any medication should never be added directly to a blood product bag.

#390 · Pharmacological and Parenteral Therapies

What are the components of total parenteral nutrition and why does it require central line administration?

Answer

Total parenteral nutrition contains a concentrated mixture of dextrose, amino acids, lipids, electrolytes, vitamins, and trace elements delivered directly into the bloodstream. Because the dextrose concentration is high, typically above 10 percent, it must be infused through a central line where rapid blood flow dilutes it and prevents damage to smaller peripheral veins. Blood glucose is monitored regularly because hyperglycemia is a common complication.

#391 · Pharmacological and Parenteral Therapies

What nursing action prevents rebound hypoglycemia when discontinuing total parenteral nutrition?

Answer

The infusion rate is tapered gradually over one to two hours rather than stopped abruptly, allowing the body's insulin response to adjust. If the bag runs out unexpectedly, a dextrose solution such as 10 percent dextrose in water is often used temporarily to prevent a sudden drop in blood glucose. Blood glucose levels are checked after discontinuation to confirm stability.

#392 · Pharmacological and Parenteral Therapies

What is the nursing priority when initiating patient controlled analgesia for postoperative pain management?

Answer

The nurse confirms the correct client, drug, concentration, and dose settings with a second regulated provider before starting the pump, since this is a high alert medication delivery system. Baseline sedation level, respiratory rate, and pain score are documented before starting and reassessed regularly during use. The client and family are taught that only the client should press the button, never a family member on the client's behalf.

#393 · Pharmacological and Parenteral Therapies

What early sign of opioid induced respiratory depression should the nurse monitor most closely in a client receiving IV opioids?

Answer

Sedation level is monitored as the earliest and most reliable indicator, because increasing drowsiness typically precedes a drop in respiratory rate. A respiratory rate below the agency threshold, often 10 to 12 breaths per minute, combined with a rising sedation score, signals a need to hold the next dose and notify the prescriber. Naloxone should be readily available whenever opioids are administered.

#394 · Pharmacological and Parenteral Therapies

How should naloxone be administered to reverse opioid induced respiratory depression without causing severe withdrawal or return of severe pain?

Answer

Naloxone is given in small titrated doses, often diluted and given slowly, to reverse respiratory depression while preserving some analgesic effect and avoiding acute withdrawal symptoms. The client is monitored closely afterward because naloxone has a shorter duration of action than many opioids, so respiratory depression can recur once it wears off. Repeat doses or a continuous infusion may be needed.

#395 · Pharmacological and Parenteral Therapies

What is the correct approach to assessing pain in a client who cannot self-report, such as a client with advanced dementia?

Answer

The nurse uses a validated behavioural pain assessment tool that scores observable indicators such as facial expression, vocalization, body movement, and changes in breathing pattern. Self-report remains the gold standard whenever a client can communicate, but behavioural tools provide an objective alternative when they cannot. Pain is reassessed after any intervention to evaluate its effectiveness.

#396 · Pharmacological and Parenteral Therapies

What are common signs of an allergic drug reaction the nurse should teach a client to report immediately?

Answer

The client should report hives, itching, swelling of the lips, tongue, or throat, wheezing, or difficulty breathing right away, since these can signal a progressing allergic reaction. Mild rash alone is monitored closely but symptoms involving the airway or breathing require emergency assessment. Any new allergy identified must be documented clearly in the client's record and communicated to the healthcare team.

#397 · Pharmacological and Parenteral Therapies

What distinguishes a Type I anaphylactic reaction from a milder allergic drug reaction, and what is the immediate nursing priority?

Answer

Anaphylaxis involves rapid onset of airway swelling, bronchospasm, and hypotension due to a massive histamine release, which can progress to shock within minutes. The immediate priority is stopping the causative drug, calling for emergency help, and administering intramuscular epinephrine as the first line treatment. Oxygen, IV fluids, and antihistamines are supportive measures given after epinephrine.

#398 · Pharmacological and Parenteral Therapies

What is the safe practice for verifying insulin doses before administration?

Answer

Insulin is classified as a high alert medication, so most agencies require an independent double check of the dose and insulin type by a second regulated provider before injection. The nurse also confirms the client's most recent blood glucose reading and checks that the insulin type matches the order, since rapid acting and long acting insulins are never interchangeable. Insulin syringes marked in units must be used rather than standard syringes to avoid dosing errors.

#399 · Pharmacological and Parenteral Therapies

Why are look alike sound alike drug names a significant medication safety concern, and how do agencies reduce this risk?

Answer

Drugs such as hydralazine and hydroxyzine, or clonidine and clonazepam, have similar names that can be confused during verbal orders or when reading handwriting, leading to serious dosing errors. Many agencies use tall man lettering, such as writing hydrOXYzine, to visually distinguish similar names on labels and computer screens. Reading back verbal orders and confirming the indication for the drug also helps catch these errors before administration.

#400 · Pharmacological and Parenteral Therapies

What is the correct nursing action when a medication error is discovered after a drug has already been given to a client?

Answer

The nurse first assesses the client for any adverse effects and notifies the prescriber promptly so appropriate monitoring or intervention can be started. The error is then documented factually in the client's record and reported through the agency's incident reporting system, without documenting the incident report itself in the chart. Being transparent with the client and family about the error, when appropriate, supports trust and follows professional accountability standards.

#401 · Reduction of Risk Potential

What is the normal adult reference range for serum potassium?

Answer

The normal adult range for serum potassium is 3.5 to 5.0 mmol/L. Values below 3.5 mmol/L indicate hypokalemia and values above 5.0 mmol/L indicate hyperkalemia. Both extremes can cause dangerous cardiac dysrhythmias, so the nurse should always check potassium against a current ECG when it is abnormal.

#402 · Reduction of Risk Potential

A client's blood pressure drops from 128/78 to 92/60 while the heart rate rises from 78 to 124. What early clinical picture does this suggest?

Answer

This combination of falling blood pressure and rising heart rate is a classic early sign of compensatory shock. The heart rate increases first to try to maintain cardiac output before blood pressure drops significantly. The nurse should recognize this trend rather than waiting for hypotension alone and should notify the primary care provider immediately.

#403 · Reduction of Risk Potential

What is the normal range for arterial blood pH?

Answer

The normal arterial blood pH range is 7.35 to 7.45. A pH below 7.35 indicates acidosis and a pH above 7.45 indicates alkalosis. This value is always interpreted together with PaCO₂ and HCO₃ to determine whether the cause is respiratory or metabolic.

#404 · Reduction of Risk Potential

During conscious sedation, which vital parameter must the nurse monitor continuously to detect respiratory depression early?

Answer

Pulse oximetry, specifically oxygen saturation, must be monitored continuously along with respiratory rate and depth. A drop in oxygen saturation below 90 percent or a respiratory rate under 8 breaths per minute signals dangerous depression and requires immediate intervention. Capnography is increasingly used because it detects hypoventilation before a drop in oxygen saturation appears.

#405 • Reduction of Risk Potential

What is the normal range for fasting blood glucose in a non-pregnant adult?

Answer

Normal fasting blood glucose is 4.0 to 6.0 mmol/L in the Canadian measurement system. A fasting value between 6.1 and 6.9 mmol/L suggests impaired fasting glucose, and 7.0 mmol/L or higher on two occasions suggests diabetes. The nurse uses this range to interpret point of care glucose checks accurately.

#406 • Reduction of Risk Potential

A postoperative client develops a temperature of 38.9 Celsius, heart rate of 118, and respiratory rate of 26 on day two after surgery. What complication should the nurse suspect first?

Answer

This combination of fever, tachycardia, and tachypnea in the early postoperative period suggests a developing surgical site infection or another source of sepsis. The nurse should assess the surgical wound, obtain blood cultures if ordered, and notify the provider promptly. Early recognition of these trending vital signs is key to preventing progression to septic shock.

#407 · Reduction of Risk Potential

What hemoglobin level in an adult female is generally considered the lower limit of normal?

Answer

For an adult female, the lower limit of normal hemoglobin is approximately 120 g/L. Values below this suggest anemia and warrant further investigation into blood loss, nutritional deficiency, or chronic disease. For adult males the lower normal limit is slightly higher at approximately 135 g/L.

#408 · Reduction of Risk Potential

What is the priority nursing action if a client receiving a blood transfusion develops chills, low back pain, and hematuria within the first 15 minutes?

Answer

The nurse must stop the transfusion immediately, keep the intravenous line open with normal saline using new tubing, and notify the provider and blood bank right away. These symptoms suggest an acute hemolytic transfusion reaction, which is life threatening if the transfusion continues. Vital signs should be reassessed frequently and the remaining blood product returned to the lab for testing.

#409 · Reduction of Risk Potential

What INR range is generally the therapeutic target for a client on warfarin for atrial fibrillation?

Answer

The therapeutic INR range for most clients on warfarin for atrial fibrillation is 2.0 to 3.0. An INR below this range increases the risk of clot formation and stroke, while an INR above this range increases the risk of bleeding. The nurse should hold the dose and notify the provider if the INR is significantly outside this range.

#410 · Reduction of Risk Potential

Which early respiratory assessment finding suggests a client is developing a pneumothorax after chest tube insertion complications?

Answer

Sudden onset of sharp chest pain with absent or diminished breath sounds on the affected side and increasing shortness of breath suggest a pneumothorax. Tracheal deviation away from the affected side and severe respiratory distress can indicate a tension pneumothorax, which is a medical emergency. The nurse should assess oxygen saturation and notify the provider immediately.

#411 · Reduction of Risk Potential

What is the normal range for serum sodium?

Answer

The normal range for serum sodium is 135 to 145 mmol/L. Hyponatremia below 135 mmol/L can cause confusion, seizures, and cerebral edema in severe cases, while hypernatremia above 145 mmol/L can cause thirst, restlessness, and dry mucous membranes. Sodium imbalances are always assessed alongside neurological status.

#412 · Reduction of Risk Potential

A client returning from a cardiac catheterization via the femoral approach reports numbness and coolness in the affected leg. What should the nurse assess first?

Answer

The nurse should immediately assess the pedal pulses, color, temperature, and capillary refill of the affected leg to check for arterial occlusion or hematoma formation at the insertion site. Loss of pulse or a cool, pale, painful limb suggests compromised circulation requiring urgent provider notification. The insertion site should also be checked for bleeding or an expanding hematoma.

#413 · Reduction of Risk Potential

What is the normal range for platelet count in an adult?

Answer

The normal adult platelet count range is 150 to 400 times 10 to the ninth per litre. A count below 150 indicates thrombocytopenia and increases bleeding risk, while a count above 400 indicates thrombocytosis and can increase clotting risk. The nurse monitors this value closely in clients on chemotherapy or with bone marrow suppression.

#414 · Reduction of Risk Potential

What early sign distinguishes compensated shock from decompensated shock?

Answer

In compensated shock, blood pressure remains near normal because the body increases heart rate and constricts peripheral vessels to maintain perfusion to vital organs. In decompensated shock, these mechanisms fail and blood pressure drops significantly, along with worsening mental status and decreased urine output. Recognizing the compensated stage allows the nurse to intervene before the client deteriorates further.

#415 · Reduction of Risk Potential

What is the normal range for serum creatinine in an adult?

Answer

The normal adult serum creatinine range is approximately 60 to 110 micromol/L, though exact values vary slightly by lab and by the client's muscle mass. Elevated creatinine suggests decreased kidney function and is used along with estimated glomerular filtration rate to stage kidney disease. The nurse should correlate creatinine trends with urine output and fluid balance.

#416 · Reduction of Risk Potential

During recovery from conscious sedation, what score or tool is commonly used to determine when a client is ready for discharge from the recovery area?

Answer

Many facilities use the Aldrete score, which evaluates activity, respiration, circulation, consciousness, and oxygen saturation to determine readiness for discharge. A higher score indicates the client has returned closer to their baseline status. The nurse should not discharge a client from monitored recovery until the required score threshold set by the facility policy is met.

#417 · Reduction of Risk Potential

What is the normal range for white blood cell count in an adult?

Answer

The normal adult white blood cell count range is approximately 4.0 to 11.0 times 10^9 per litre. A count above this range, called leukocytosis, often indicates infection or inflammation, while a count below this range, called leukopenia, can indicate bone marrow suppression or certain viral infections. The nurse interprets this value alongside fever and other clinical signs.

#418 · Reduction of Risk Potential

A client following abdominal surgery has a rigid, board like abdomen with rebound tenderness and absent bowel sounds. What complication does this suggest?

Answer

These findings suggest peritonitis, which can result from anastomotic leak, perforation, or infection following abdominal surgery. This is a surgical emergency requiring immediate provider notification. The nurse should also monitor for fever, tachycardia, and hypotension, which often accompany this complication.

#419 · Reduction of Risk Potential

What is the normal range for oxygen saturation measured by pulse oximetry in a healthy adult?

Answer

Normal oxygen saturation by pulse oximetry is 95 to 100 percent in a healthy adult without underlying lung disease. Saturation below 90 percent indicates significant hypoxemia and requires prompt intervention. Clients with chronic lung disease may have a different individualized target range set by their provider.

#420 · Reduction of Risk Potential

What clinical trend in urine output over several hours suggests a client is going into acute kidney injury or worsening shock?

Answer

Urine output falling below 0.5 mL per kilogram of body weight per hour for several consecutive hours suggests inadequate renal perfusion, which can occur in shock or acute kidney injury. This oliguria should be reported promptly along with trends in blood pressure and creatinine. The nurse should also review recent fluid intake and any nephrotoxic medications the client has received.

#421 · Reduction of Risk Potential

What is the normal range for serum calcium in an adult?

Answer

The normal adult total serum calcium range is approximately 2.10 to 2.60 mmol/L. Hypocalcemia below this range can cause muscle cramps, tetany, and a positive Chvostek or Trousseau sign, while hypercalcemia above this range can cause confusion, weakness, and cardiac dysrhythmias. Ionized calcium is a more precise measure and is used when albumin levels are abnormal.

#422 · Reduction of Risk Potential

A client with a central venous catheter develops sudden dyspnea, chest pain, and a drop in oxygen saturation during a dressing change. What complication should the nurse suspect?

Answer

These symptoms suggest an air embolism, which can occur if the catheter hub is open to air during a dressing or tubing change. The nurse should immediately clamp the catheter, place the client in the left lateral Trendelenburg position, administer oxygen, and call for emergency assistance. This position helps trap the air in the right atrium away from the pulmonary outflow tract.

#423 · Reduction of Risk Potential

What is the normal range for PaCO₂ in an arterial blood gas?

Answer

The normal range for PaCO₂ is 35 to 45 mmHg. A value above 45 mmHg indicates hypercapnia and respiratory acidosis if uncompensated, while a value below 35 mmHg indicates hypocapnia and respiratory alkalosis if uncompensated. This value reflects how effectively the client is ventilating.

#424 · Reduction of Risk Potential

What early vital sign changes should alert the nurse to developing sepsis in a hospitalized client?

Answer

Early signs of sepsis include a temperature above 38 Celsius or below 36 Celsius, heart rate above 90 beats per minute, respiratory rate above 20 breaths per minute, and altered mental status. These changes often appear before a significant drop in blood pressure occurs. The nurse should escalate care promptly since early treatment with fluids and antibiotics improves outcomes significantly.

#425 · Reduction of Risk Potential

What is the normal therapeutic range for serum digoxin level?

Answer

The normal therapeutic range for serum digoxin is approximately 0.8 to 2.0 nanograms per millilitre, though many providers now target the lower end of this range to reduce toxicity risk. Signs of digoxin toxicity include nausea, visual disturbances such as seeing yellow or green halos, and dysrhythmias. The nurse should check the apical pulse before administering digoxin and hold the dose if it is below 60 beats per minute.

#426 · Reduction of Risk Potential

A client on a magnesium sulfate infusion for preeclampsia develops absent deep tendon reflexes and a respiratory rate of 10. What should the nurse do first?

Answer

The nurse should stop the magnesium sulfate infusion immediately because these findings indicate magnesium toxicity. The nurse should then notify the provider, prepare to administer calcium gluconate as the antidote, and continue close monitoring of respiratory rate, reflexes, and level of consciousness. Serum magnesium level should also be drawn to confirm the toxic level.

#427 · Reduction of Risk Potential

What is the normal range for a resting adult respiratory rate?

Answer

The normal resting adult respiratory rate is 12 to 20 breaths per minute. A sustained rate above 20 breaths per minute, called tachypnea, can indicate pain, anxiety, fever, or respiratory or metabolic compromise. A sustained rate below 12 breaths per minute, called bradypnea, can indicate opioid overdose or worsening neurological status.

#428 · Reduction of Risk Potential

Following a liver biopsy, what position and monitoring does the nurse implement to reduce the risk of bleeding complications?

Answer

The nurse positions the client on their right side with a small pillow or rolled towel under the biopsy site for at least the first hour to apply pressure and reduce bleeding risk. Vital signs are monitored frequently, and the nurse watches for signs of internal bleeding such as falling blood pressure, rising heart rate, and increasing abdominal pain or distension. The client is kept on bed rest for several hours per facility protocol.

#429 · Reduction of Risk Potential

What is the earliest sign of internal hemorrhage after abdominal surgery?

Answer

Tachycardia is usually the earliest sign, occurring as the heart compensates for falling blood volume before blood pressure drops. A drop in blood pressure is a later and more ominous finding. The nurse should also watch for restlessness, pallor, and a rising respiratory rate.

#430 · Reduction of Risk Potential

A client develops sudden sharp chest pain and shortness of breath three days after hip surgery. What complication should the nurse suspect first?

Answer

Pulmonary embolism should be suspected first, since immobility after hip surgery is a major risk factor for deep vein thrombosis that can dislodge and travel to the lungs. Other signs include tachycardia, apprehension, and a drop in oxygen saturation. The nurse should raise the head of the bed, apply oxygen, and call for help immediately.

#431 · Reduction of Risk Potential

What clinical findings suggest a deep vein thrombosis in the calf?

Answer

Unilateral leg swelling, warmth, redness, and calf pain or tenderness suggest a deep vein thrombosis. The affected leg is often larger in circumference than the unaffected leg when measured at the same point. The nurse should never massage the area, since this can dislodge the clot.

#432 · Reduction of Risk Potential

What does the nurse see when a surgical wound dehisces?

Answer

Wound dehiscence is the separation of the surgical incision edges, often revealing underlying tissue. The client may report a sudden feeling that something has given way, sometimes with a burst of serosanguineous drainage. If the intestines protrude through the opening, this is evisceration and is a surgical emergency.

#433 · Reduction of Risk Potential

What should the nurse do immediately if a client's surgical wound eviscerates?

Answer

The nurse should cover the exposed organs with a sterile saline soaked dressing, place the client in low Fowler's position with knees bent, and remain with the client while calling for immediate help. The nurse should not attempt to push the organs back in and should keep the client calm and on nothing by mouth in case of return to surgery.

#434 · Reduction of Risk Potential

What score range on the Glasgow Coma Scale indicates a severe brain injury?

Answer

A score of 8 or lower on the Glasgow Coma Scale indicates a severe brain injury and usually means the client cannot protect their own airway. The scale ranges from 3, which is deep coma, to 15, which is fully alert. Scores from 9 to 12 indicate moderate injury and 13 to 15 indicate mild injury.

#435 · Reduction of Risk Potential

What three areas of function does the Glasgow Coma Scale assess?

Answer

The Glasgow Coma Scale assesses eye opening, verbal response, and motor response. Each category is scored separately and the three scores are added together for a total ranging from 3 to 15. A falling total score over time is a critical sign of neurological decline.

#436 · Reduction of Risk Potential

What does a low score on the Braden scale indicate about a client's risk?

Answer

A low Braden scale score indicates a higher risk of developing a pressure injury. The scale scores sensory perception, moisture, activity, mobility, nutrition, and friction and shear, with lower totals meaning greater risk. A total score of 18 or below is generally considered at risk in most adult populations.

#437 · Reduction of Risk Potential

Which six categories make up the Braden scale for predicting pressure injury risk?

Answer

The six categories are sensory perception, moisture, activity, mobility, nutrition, and friction and shear. Each category except friction and shear is scored from 1 to 4, and friction and shear is scored from 1 to 3. The lowest possible total score indicates the highest risk for skin breakdown.

#438 · Reduction of Risk Potential

What five signs does the Apgar score measure in a newborn?

Answer

The Apgar score measures heart rate, respiratory effort, muscle tone, reflex irritability, and color. Each sign is scored 0, 1, or 2, giving a total from 0 to 10. The score is usually assessed at one minute and again at five minutes after birth.

#439 · Reduction of Risk Potential

What does an Apgar score of 7 to 10 at five minutes mean for a newborn?

Answer

A score of 7 to 10 means the newborn is in good condition and requires only routine care. A score of 4 to 6 suggests moderate difficulty adjusting to extrauterine life and may need stimulation or oxygen. A score of 0 to 3 indicates severe distress and requires immediate resuscitation.

#440 · Reduction of Risk Potential

What should the nurse monitor most closely during moderate conscious sedation?

Answer

The nurse should monitor the client's level of consciousness, respiratory rate, and oxygen saturation continuously throughout the procedure. Blood pressure and heart rate are also checked at frequent set intervals. Equipment for airway management and reversal agents should be at the bedside before the sedation begins.

#441 · Reduction of Risk Potential

What is the priority nursing action if a client becomes unresponsive to verbal stimuli during conscious sedation?

Answer

The priority action is to assess the airway, breathing, and oxygen saturation right away, since over-sedation can suppress respiratory drive. The nurse should stop administering the sedating agent and prepare to give a reversal agent if ordered. The care provider performing the procedure must be notified immediately.

#442 · Reduction of Risk Potential

Why must a client remain nothing by mouth for a set period before conscious sedation?

Answer

Fasting reduces the risk of vomiting and aspiration while the client's protective airway reflexes are dulled by the sedating medication. Aspiration of stomach contents into the lungs can cause a serious pneumonia or airway obstruction. The exact fasting time depends on the type of food or liquid last consumed.

#443 · Reduction of Risk Potential

A client's potassium level returns at 6.2 mmol/L. What should the nurse do first?

Answer

The nurse should first check the client's cardiac rhythm and notify the care provider, since hyperkalemia at this level can cause dangerous dysrhythmias including peaked T waves and cardiac arrest. The nurse should also hold any potassium supplements or potassium-sparing medications until further orders. This value is well above the normal range of about 3.5 to 5.0 mmol/L.

#444 · Reduction of Risk Potential

What symptoms would the nurse expect in a client with a sodium level of 120 mmol/L?

Answer

A sodium level of 120 mmol/L is significantly below the normal range of about 135 to 145 mmol/L and would likely cause confusion, headache, nausea, and muscle weakness. In severe cases the client may develop seizures or a decreased level of consciousness from cerebral swelling. The nurse should monitor neurological status closely and report the value promptly.

#445 · Reduction of Risk Potential

What is the normal range for a client's international normalized ratio while not on anticoagulant therapy?

Answer

The normal international normalized ratio for a client not on anticoagulant therapy is approximately 0.8 to 1.1. Clients on warfarin therapy for conditions like atrial fibrillation are usually kept in a therapeutic range of about 2.0 to 3.0. A value above the therapeutic range increases bleeding risk.

#446 · Reduction of Risk Potential

What lab value is used to monitor a client receiving continuous intravenous heparin?

Answer

Activated partial thromboplastin time is used to monitor clients on continuous intravenous heparin. The therapeutic range is usually about one and a half to two and a half times the normal control value. The dose is adjusted based on this result to keep the client anticoagulated without excessive bleeding risk.

#447 · Reduction of Risk Potential

What does an elevated blood urea nitrogen and creatinine together usually indicate?

Answer

An elevated blood urea nitrogen together with an elevated creatinine usually indicates impaired kidney function, since both substances are normally filtered and excreted by the kidneys. Creatinine is a more specific marker of kidney function because blood urea nitrogen can also rise with dehydration or high protein intake alone. The nurse should assess urine output and fluid status when these values are elevated together.

#448 · Reduction of Risk Potential

A postoperative client's oxygen saturation drops to 88% on room air. What should the nurse do first?

Answer

The nurse should first apply supplemental oxygen and reassess the client's respiratory rate, effort, and level of consciousness. A saturation of 88% is below the acceptable range, which is generally 95% or higher for most adults. The nurse should then notify the care provider if the saturation does not improve with oxygen.

#449 · Reduction of Risk Potential

What vital sign pattern is classic for early compensated hypovolemic shock?

Answer

Early compensated hypovolemic shock typically presents with tachycardia and a normal or only slightly low blood pressure, since the body constricts peripheral vessels to maintain perfusion to vital organs. The client may also appear anxious with cool, pale skin. Blood pressure usually does not drop significantly until the client has lost a large volume of blood, so waiting for hypotension can delay recognition.

#450 · Reduction of Risk Potential

What findings suggest compartment syndrome after a client has a cast applied to a fractured limb?

Answer

Compartment syndrome presents with severe pain that is out of proportion to the injury and does not improve with prescribed analgesics. Additional signs include pallor, decreased pulses, paresthesia, and pain with passive stretching of the affected muscles. This is a surgical emergency because prolonged pressure can cause permanent nerve and muscle damage.

#451 · Reduction of Risk Potential

Why does the nurse auscultate bowel sounds before advancing a postoperative client's diet?

Answer

The nurse listens for active bowel sounds to confirm that peristalsis has returned after anesthesia and abdominal surgery temporarily slow gut motility. Advancing the diet before bowel function returns can cause nausea, vomiting, or abdominal distension. The nurse should also ask about flatus or a first bowel movement as further evidence that the bowel is working again.

#452 · Reduction of Risk Potential

What is the priority nursing assessment in the first hours after a client returns from a cardiac catheterization with a femoral access site?

Answer

The priority assessment is the femoral insertion site for bleeding or hematoma formation, along with checking distal pulses, color, and temperature of the affected limb. The client is usually kept on bed rest with the affected leg straight for a prescribed period to reduce bleeding risk. Any expanding swelling or sudden back pain should be reported immediately, since it can signal retroperitoneal bleeding.

#453 · Reduction of Risk Potential

What early sign should the nurse watch for after removal of a chest tube?

Answer

The nurse should watch for signs of respiratory distress such as increased respiratory rate, shortness of breath, or asymmetrical chest movement, which can indicate a recurring pneumothorax. The occlusive dressing over the site should be checked to ensure it remains airtight. A chest x-ray is often ordered after removal to confirm the lung remains expanded.

#454 · Reduction of Risk Potential

What blood glucose value defines hypoglycemia in most adult clients, and what is the priority action?

Answer

A blood glucose below 4.0 mmol/L generally defines hypoglycemia in adult clients. If the client is conscious and able to swallow, the priority action is to give a fast-acting source of glucose such as juice or glucose tablets, then recheck the blood glucose in fifteen minutes. If the client is unconscious, the nurse should not give anything by mouth and should prepare to administer glucagon or intravenous dextrose instead.

#455 · Reduction of Risk Potential

What assessment finding after thyroid surgery could indicate damage to the parathyroid glands?

Answer

Tingling around the mouth and fingertips, along with muscle twitching or spasms, can indicate hypocalcemia from accidental removal or damage to the parathyroid glands during thyroid surgery. The nurse can check for Trousseau's sign or Chvostek's sign to further assess for low calcium. Calcium levels should be monitored closely in the postoperative period.

#456 · Physiological Adaptation

What are the classic early signs of left-sided heart failure?

Answer

Left-sided heart failure causes blood to back up into the pulmonary circulation, producing dyspnea, orthopnea, crackles on lung auscultation, and a dry cough. Fatigue and activity intolerance are also common because the left ventricle cannot pump enough blood forward to meet the body's needs.

#457 · Physiological Adaptation

What are the classic signs of right-sided heart failure?

Answer

Right-sided heart failure causes blood to back up into the systemic venous system, leading to jugular vein distension, peripheral or dependent edema, hepatomegaly, and rapid weight gain. Ascites can also develop as fluid accumulates in the abdominal cavity.

#458 · Physiological Adaptation

What ECG changes indicate an acute ST-elevation myocardial infarction (STEMI)?

Answer

A STEMI shows ST-segment elevation in the leads facing the area of infarction, often with reciprocal ST depression in opposite leads. New Q waves can develop later as the tissue becomes necrotic, and T waves may become peaked early then invert as the injury evolves.

#459 · Physiological Adaptation

A client reports crushing substernal chest pain radiating to the left arm and jaw with diaphoresis and nausea. What is the priority nursing action?

Answer

The practical nurse should obtain a 12-lead ECG and vital signs immediately and notify the registered nurse or physician so cardiac markers and treatment can begin without delay. Oxygen is applied only if the client is hypoxic, and the client is kept on bed rest with continuous cardiac monitoring.

#460 · Physiological Adaptation

What are the four core components of the MONA acronym used in early myocardial infarction management?

Answer

MONA stands for morphine, oxygen, nitroglycerin, and acetylsalicylic acid (ASA). Oxygen is given only if oxygen saturation is low, and these interventions aim to relieve pain, reduce cardiac workload, and limit further clot formation while definitive treatment is arranged.

#461 · Physiological Adaptation

What ECG rhythm shows no identifiable P waves, an irregularly irregular rhythm, and a chaotic baseline?

Answer

This description matches atrial fibrillation, in which the atria quiver instead of contracting effectively. The main risks are stroke from atrial thrombus formation and a rapid ventricular response that can compromise cardiac output, so anticoagulation and rate control are common treatment goals.

#462 · Physiological Adaptation

What is the immediate nursing priority for a client found in pulseless ventricular tachycardia?

Answer

This is a lethal arrhythmia requiring immediate cardiopulmonary resuscitation and defibrillation as part of the code team response. The practical nurse should call for help, start chest compressions, and ensure the defibrillator and emergency cart arrive as quickly as possible.

#463 · Physiological Adaptation

What serum potassium level is considered severe hyperkalemia, and what ECG change is an early warning sign?

Answer

A serum potassium level above 6.5 mmol/L is considered severe hyperkalemia. An early ECG warning sign is tall, peaked T waves, which can progress to a widened QRS complex and eventually a fatal arrhythmia if untreated.

#464 · Physiological Adaptation

What are the priority treatments for severe symptomatic hyperkalemia?

Answer

Calcium gluconate is given first to stabilize the cardiac cell membrane and protect the heart, even though it does not lower the potassium level itself. Insulin with dextrose, sodium bicarbonate, and beta-agonists such as salbutamol shift potassium into the cells, while sodium polystyrene sulfonate or dialysis actually removes potassium from the body.

#465 · Physiological Adaptation

What are the signs and symptoms of hypokalemia?

Answer

Hypokalemia causes muscle weakness, leg cramps, fatigue, and decreased bowel motility that can progress to paralytic ileus. On the ECG, it can cause flattened T waves, a prominent U wave, and increases the risk of dangerous ventricular arrhythmias, especially in clients taking digoxin.

#466 · Physiological Adaptation

Why must intravenous potassium chloride never be given as an undiluted IV push?

Answer

Undiluted or rapidly infused potassium chloride can cause immediate cardiac arrest because it drastically alters the electrical activity of the heart. It must always be diluted appropriately and infused slowly using an infusion pump with continuous cardiac monitoring.

#467 · Physiological Adaptation

What are the clinical signs of hyponatremia?

Answer

Hyponatremia causes cellular swelling as water shifts into cells, leading to headache, confusion, lethargy, and muscle cramps. In severe or rapidly developing cases it can cause seizures and cerebral edema because brain cells are especially sensitive to the drop in sodium concentration.

#468 · Physiological Adaptation

Why must severe hyponatremia be corrected slowly rather than rapidly?

Answer

Correcting sodium too quickly can cause osmotic demyelination syndrome, a serious and often permanent injury to the nerve cells in the brainstem. Sodium levels are raised gradually with frequent monitoring to allow brain cells to adjust safely.

#469 · Physiological Adaptation

What are the classic signs of hypocalcemia, and what two bedside tests can help confirm it?

Answer

Hypocalcemia causes numbness and tingling around the mouth and fingertips along with muscle cramps and tetany. Trousseau's sign, a hand spasm after inflating a blood pressure cuff, and Chvostek's sign, a facial twitch after tapping the facial nerve, are two bedside tests used to help confirm low calcium.

#470 · Physiological Adaptation

What is the pathophysiology behind the edema seen in nephrotic syndrome?

Answer

Nephrotic syndrome causes the glomeruli to leak large amounts of protein into the urine, leading to low serum albumin. This drop in oncotic pressure allows fluid to shift out of the blood vessels into the tissues, producing widespread edema, especially around the eyes and in the legs.

#471 · Physiological Adaptation

What is the difference between prerenal, intrarenal, and postrenal acute kidney injury?

Answer

Prerenal acute kidney injury is caused by reduced blood flow to the kidneys, such as from severe dehydration or hemorrhage. Intrarenal injury results from direct damage to kidney tissue, such as acute tubular necrosis, while postrenal injury results from an obstruction to urine outflow, such as a blocked catheter or an enlarged prostate.

#472 · Physiological Adaptation

What dietary restrictions are typically taught to a client with chronic kidney disease on hemodialysis?

Answer

Clients are usually taught to limit potassium, phosphorus, sodium, and fluid intake because failing kidneys cannot excrete these effectively between dialysis sessions. Protein intake is often adjusted to a moderate, high-biological-value amount to reduce waste production while still preventing malnutrition.

#473 · Physiological Adaptation

What is the pathophysiology of diabetic ketoacidosis (DKA)?

Answer

DKA occurs when a severe lack of insulin forces the body to break down fat for energy, producing ketones that lower the blood pH and cause metabolic acidosis. The high blood glucose also causes osmotic diuresis, leading to significant dehydration and electrolyte loss, particularly potassium.

#474 · Physiological Adaptation

What is the priority initial treatment for a client presenting in diabetic ketoacidosis?

Answer

The priority is intravenous fluid replacement with isotonic saline to correct dehydration and improve tissue perfusion before insulin therapy begins. Once fluids are underway, a continuous intravenous insulin infusion is started, and potassium levels are monitored closely since insulin drives potassium into the cells.

#475 · Physiological Adaptation

What are the early warning signs of increased intracranial pressure?

Answer

Early signs include a change in level of consciousness, restlessness or irritability, headache, and vomiting that may occur without nausea. As pressure continues to rise, pupils may become sluggish or unequal and vital signs can show Cushing's triad of widening pulse pressure, bradycardia, and irregular respirations.

#476 · Physiological Adaptation

What is the difference between an ischemic stroke and a hemorrhagic stroke?

Answer

An ischemic stroke occurs when a clot blocks blood flow to part of the brain, cutting off oxygen and nutrients to that tissue. A hemorrhagic stroke occurs when a blood vessel in the brain ruptures and bleeds, which also increases intracranial pressure and can rapidly worsen neurological status.

#477 · Physiological Adaptation

What does the FAST acronym stand for in recognizing stroke symptoms?

Answer

FAST stands for face drooping, arm weakness, speech difficulty, and time to call for emergency help. This tool helps identify stroke quickly so treatment can begin within the critical window when clot-busting medication may still be effective.

#478 · Physiological Adaptation

What are the priority assessment findings for a client with suspected upper gastrointestinal bleeding?

Answer

The nurse should watch for hematemesis, which is vomiting of blood that may look bright red or like coffee grounds, and melena, which is black tarry stool. Vital sign changes such as tachycardia and hypotension can indicate significant blood loss and the beginning of hypovolemic shock.

#479 · Physiological Adaptation

What early sign of anaphylaxis should a practical nurse watch for after a client receives a new medication or blood product?

Answer

Sudden onset of hives, itching, throat tightness, or difficulty breathing signals anaphylaxis. Stridor and facial or tongue swelling indicate airway compromise and require immediate action. The nurse should stop the causative agent right away and call for help.

#480 · Physiological Adaptation

What is the first drug given for anaphylaxis and by what route?

Answer

Intramuscular epinephrine, usually into the anterolateral thigh, is the first line treatment. It reverses bronchospasm and vasodilation faster than other medications. Antihistamines and corticosteroids are given afterward as adjunct therapy, not as the initial drug.

#481 · Physiological Adaptation

A client with COPD has an oxygen saturation of 88 percent. Why should the nurse avoid raising oxygen flow above the prescribed low rate?

Answer

Many clients with chronic COPD rely on hypoxia rather than hypercapnia to drive breathing. Giving high flow oxygen can blunt this drive and cause respiratory depression. Oxygen should be titrated carefully to a target saturation, often around 88 to 92 percent, and increases should be gradual with close monitoring.

#482 · Physiological Adaptation

What position helps a client experiencing an acute asthma exacerbation breathe more easily?

Answer

A high Fowler or tripod position, sitting upright and leaning slightly forward with arms supported, maximizes lung expansion. This position reduces the work of breathing by allowing accessory muscles to assist ventilation. The nurse should also apply prescribed bronchodilators and monitor oxygen saturation.

#483 · Physiological Adaptation

Which finding during an asthma attack suggests the client is worsening rather than improving, even if wheezing seems quieter?

Answer

A silent chest with minimal air movement is a serious warning sign, not a sign of improvement, because it means airflow is too restricted to produce wheezing. Rising respiratory rate followed by fatigue and a falling respiratory rate can also signal impending respiratory failure. The nurse must escalate care immediately if breath sounds diminish this way.

#484 · Physiological Adaptation

What are the classic warning signs of an ischemic stroke that a nurse should teach clients to recognize using the FAST acronym?

Answer

FAST stands for facial drooping, arm weakness, speech difficulty, and time to call emergency services. Sudden onset of these symptoms means every minute counts because brain tissue is dying without adequate blood flow. Rapid recognition and transport improve the chance of receiving clot-dissolving treatment within the window of benefit.

#485 · Physiological Adaptation

Why is blood glucose checked before giving thrombolytic therapy to a client with suspected stroke?

Answer

Severe hypoglycemia can mimic stroke symptoms such as confusion, weakness, and slurred speech. Correcting a low blood glucose may resolve the symptoms without need for thrombolytics. Ruling this out prevents giving a clot buster to someone who does not actually have a stroke.

#486 · Physiological Adaptation

What are the early signs of increased intracranial pressure the nurse should report immediately?

Answer

Early signs include a change in level of consciousness, restlessness, headache, and pupil changes. A decreasing level of consciousness is often the earliest and most reliable indicator. Waiting for Cushing triad, which includes widening pulse pressure, bradycardia, and irregular respirations, means damage is already advanced.

#487 · Physiological Adaptation

What positioning should a nurse maintain for a client at risk of increased intracranial pressure?

Answer

The head of the bed should be elevated to about 30 degrees with the head kept in neutral alignment. This position promotes venous drainage from the brain and helps lower intracranial pressure. Extreme hip or neck flexion should be avoided since it can impede venous outflow and raise pressure further.

#488 · Physiological Adaptation

What is the priority nursing action during a client's active tonic-clonic seizure?

Answer

The priority is protecting the client from injury by easing them to the floor if needed, clearing the area of hazards, and turning them onto their side. The nurse should never place anything in the client's mouth or attempt to restrain the movements. Timing the seizure and observing its features helps guide further care.

#489 · Physiological Adaptation

What defines status epilepticus and why is it a medical emergency?

Answer

Status epilepticus is a seizure lasting longer than five minutes or repeated seizures without full recovery of consciousness between them. Prolonged seizure activity can cause hypoxia, hyperthermia, and permanent brain injury. It requires urgent intravenous benzodiazepine treatment and airway management.

#490 · Physiological Adaptation

What laboratory and clinical findings confirm diabetic ketoacidosis?

Answer

DKA is characterized by blood glucose typically above 14 mmol/L, ketones in the blood or urine, and metabolic acidosis with a low serum bicarbonate. Clients often show deep rapid Kussmaul breathing, fruity breath odor, and dehydration. It occurs most often in type 1 diabetes but can also occur in type 2 diabetes, especially during severe illness or with SGLT2 inhibitor use.

#491 · Physiological Adaptation

What is the first treatment priority for a client presenting in diabetic ketoacidosis?

Answer

Fluid resuscitation with intravenous isotonic saline comes first to correct dehydration and improve perfusion. Insulin therapy follows to reduce blood glucose and stop ketone production. Potassium levels must be checked before and during insulin administration since insulin drives potassium into cells and can cause dangerous hypokalemia.

#492 · Physiological Adaptation

How does hyperosmolar hyperglycemic state differ from diabetic ketoacidosis?

Answer

Hyperosmolar hyperglycemic state involves extremely high blood glucose, often above 33 mmol/L, with minimal or no ketones because some insulin is still present. It typically occurs in older clients with type 2 diabetes and develops more slowly than DKA. Severe dehydration and altered mental status are prominent features requiring aggressive fluid replacement.

#493 · Physiological Adaptation

What signs indicate acute kidney injury in a hospitalized client?

Answer

Signs include a sudden drop in urine output, rising serum creatinine and blood urea nitrogen, and fluid retention with edema. Electrolyte disturbances such as hyperkalemia are common because the kidneys cannot excrete potassium normally. The nurse should track intake and output closely and report oliguria promptly.

#494 · Physiological Adaptation

Why is hyperkalemia particularly dangerous in a client with acute kidney injury?

Answer

Elevated potassium alters the electrical activity of the heart and can cause peaked T waves, widened QRS complexes, and life threatening dysrhythmias including cardiac arrest. The kidneys normally excrete excess potassium, so impaired kidney function allows it to accumulate quickly. Treatment may include calcium gluconate, insulin with glucose, and dialysis for severe cases.

#495 · Physiological Adaptation

What dietary and fluid teaching applies to a client with chronic kidney disease?

Answer

Clients are usually taught to limit sodium, potassium, and phosphorus while monitoring fluid intake closely to avoid overload. Protein intake may also be adjusted based on the stage of disease and dialysis status. The nurse should reinforce reading food labels and recognizing hidden sources of potassium such as bananas, oranges, and salt substitutes.

#496 · Physiological Adaptation

What are the priority assessment findings in a client with an upper gastrointestinal bleed?

Answer

Key findings include hematemesis or coffee ground emesis, melena, tachycardia, and hypotension suggesting hypovolemia. The nurse should monitor vital signs frequently and assess for signs of shock as blood volume drops. Hemoglobin and hematocrit trends help track the severity and response to treatment.

#497 · Physiological Adaptation

Why does melena, a black tarry stool, occur with an upper gastrointestinal bleed rather than bright red blood?

Answer

Blood from the upper GI tract is exposed to digestive enzymes and stomach acid as it passes through the intestines. This exposure breaks down hemoglobin and turns the stool black and tarry with a distinct odor. Bright red blood in stool more often indicates bleeding lower in the GI tract, closer to the rectum.

#498 · Physiological Adaptation

What are the classic clinical signs used to identify sepsis in a client?

Answer

Signs include fever or hypothermia, tachycardia, tachypnea, and altered mental status, often alongside a known or suspected infection source. As sepsis worsens, blood pressure can drop and organs may begin to fail, indicating septic shock. Early recognition and prompt antibiotics and fluids improve survival.

#499 · Physiological Adaptation

What is the recommended time frame for administering antibiotics once sepsis is suspected?

Answer

Antibiotics should be started as soon as possible, ideally within one hour of recognizing sepsis, after blood cultures are drawn. Delayed antibiotic administration is linked to significantly higher mortality. Intravenous fluid resuscitation is typically started at the same time to support blood pressure and perfusion.

#500 · Physiological Adaptation

How does hypovolemic shock differ from cardiogenic shock in terms of underlying cause?

Answer

Hypovolemic shock results from a significant loss of blood or fluid volume, such as hemorrhage or severe dehydration, reducing the amount of blood available to circulate. Cardiogenic shock occurs when the heart itself fails to pump effectively, even though fluid volume may be normal. Both types cause poor tissue perfusion but require different treatment approaches, fluid replacement versus improving cardiac function.